

Figure 2: Survey questions asked to participants pre and post deliberation

AMR Pre-Deliberation Survey

Q1: Overall, on a 0 to 10 scale, where 0 is extremely difficult, 10 is not at all difficult, and 5 is in the middle, how difficult would you say it is for you to access the **medicine** you need when you need it?

0	1	2	3	4	5	6	7	8	9	10
0 = Extremely Difficult						10 = Not at all Difficult				

Q2: And, how difficult would you say it is for you to access the **antibiotic** you need when you need it? An antibiotic is a medicine that prevents or treats a bacterial infection.

0	1	2	3	4	5	6	7	8	9	10
0 = Extremely Difficult						10 = Not at all Difficult				

Q3: Are you a farmer or someone who keeps animals at home?

Yes	CONTINUE
No	SKIP TO Q6

Q4: If yes, on a 0 to 10 scale, where 0 is extremely difficult, 10 is not at all difficult, how difficult is it for you to access **medicine** for your animals?

0	1	2	3	4	5	6	7	8	9	10
0 = Extremely Difficult						10 = Not at all Difficult				

Q5: And, how difficult is it for you to access **antibiotics** for your animals?

0	1	2	3	4	5	6	7	8	9	10
0 = Extremely Difficult						10 = Not at all Difficult				

Now, we're going to ask you about some public policy proposals about **access to antibiotics**. Not everyone has thought much about these, so if you don't have an opinion or haven't thought much about it, please feel free to say, 'don't know'.

Q6. On a 0 to 10 scale, where 0 is strongly oppose, 10 is strongly support, and 5 is in the middle, how strongly would you oppose or support the following?
Here, we use 'basic oral antibiotics' to mean antibiotics that would be first choice of most doctors for common infections acquired in the community (that is not in a hospital).

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
Governments in low- and middle-income countries should ensure appropriate access to antibiotics for all citizens including those who have difficulty in accessing the health system	1											
Governments in low- and middle-income countries should increase access to antibiotics to support all farmers for their animals, including those who have little access to veterinarians	2											
Governments should develop clear guidelines and support for controlled <u>informal</u> dispensing and monitoring of the most widely needed basic oral antibiotics <u>without</u> a doctor's or veterinarian's prescription.	3											
Registered pharmacists should be able to dispense basic oral antibiotics over the counter for some community-acquired infections without a prescription.	4											
Informal antibiotic sellers such as Patent and Proprietary Medicine Vendors and informal drug shops should be regulated and monitored to dispense a limited number of basic oral antibiotics for specific infections under strict guidelines but still without a prescription.	5											
Veterinarian assistants or allied veterinary professionals should be allowed to dispense antibiotics to farmers for a limited number of infections in livestock without a prescription.	6											
Governments should develop monitoring (and evaluation) mechanisms to include informal sales of antibiotics.	7											
Governments should report the volumes of sales from these informal sources annually to the Global Antibiotic Surveillance System (a global monitoring system)	8											

set up by the World Health Organization)												
Governments should incentivise informal sellers to use only quality-assured antibiotics.	9											
An international initiative is needed to ensure affordable steady supply of quality-assured basic oral antibiotics in the low- and middle-income countries including local manufacturing.	10											

Q7. Now, on the same scale, the following proposals are about **making the best use of antibiotics**. Some proposals refer to a term called ‘point of care’, which means done by the patient in the doctor’s rooms or at the bedside without needing to send a sample away to a laboratory.

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
An international initiative is needed to ensure availability of proven/regulated point of care diagnostic tests to support appropriate antibiotic use in all countries	1											
Governments should make currently available validated point of care, rapid diagnostic tests available at every primary health outlet.	2											
Point of care rapid diagnostic tests need to be available and a part of care pathways and should be reimbursed by the State or Medical Insurance Companies.	3											
There should be national certification standards from the manufacture of antibiotics in all countries for safe waste/outflow disposal, with penalties if standards are not met.	4											

Q8: And, on the same scale, how about the following proposals on **infection prevention**?

		0	1	2	3	4	5	6	7	8	9	10	
		0 = Strongly Oppose						10 = Strongly Support					
National governments should prioritize infection prevention tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases.	1												
Everyone should have access to clean water and safe sanitation (WASH) in their community and health system.	2												
Specific government funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene.	3												
Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage, to capture all children.	4												
An adult programme of vaccination should be created by national governments to ensure vaccination of vulnerable populations for influenza and pneumonia.	5												
Influenza vaccination should be incentivized for patient-facing healthcare workers	6												
Hospital accreditation should be linked to infection prevention, such as using hand washing rates as a factor.	7												
Hospitals should be mandated to publish their hand hygiene rates annually.	8												
Farms should ensure good infection prevention and biosecurity	9												
Farming, including fish, should use vaccination	10												
Farm workers should be taught hand hygiene	11												

Q9. And, here are some proposals about **food security**. Still on the same 0 to 10 scale.

In some of the proposals that follow, the term 'routine' antibiotics is used. This refers to the use of antibiotics for animal growth promotion and wide scale prevention of infection when there is no evidence of infection in the herd or flock.

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
An identifying label should accompany food that is produced without the use of routine antibiotics	1											
Large-scale food suppliers – supermarkets, restaurant chains, intensive farming etc., – should be incentivized to promote the use of food produced without routine antibiotics.	2											
Large-scale food suppliers – supermarkets, restaurant chains, intensive farming etc., – should be regulated to promote the use of food produced without routine antibiotics.	3											
Governments should regulate farmers to establish infection prevention programs on farms (including vaccination) linked to an inspection program.	4											
Group prevention therapy with antibiotics for sick animals should be under the management of a veterinarian or allied professional.	5											
Governments should ban from food production systems the use of antibiotics that are critically important for humans.	6											

Q10. And, now, here are proposals about **awareness and education**. Still on the same 0 to 10 scale.

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
Food produced with the routine use of antibiotics should be identified by an international standard, such as a hazard-warning symbol or label.	1											
Hand hygiene and infection prevention should be part of the standard curriculum in schools from the start of schooling through to graduation.	2											

Awareness campaigns should focus on the risks and harms of taking <u>unnecessary</u> antibiotics on humans, food security, and the environment.	3											
National governments should increase funding for awareness campaigns into vaccine hesitancy and improving understanding about people's behaviours relating to vaccination.	4											

Q11. Now, these proposals are regarding targets for antibiotics. Still on the same 0 to 10 scale.

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
There should be global targets to address bacterial resistance to antibiotics, just like global targets for climate.	1											
There should be a global target to reduce human deaths from antibiotic resistance.	2											
There should be a global target to stop the use of medically critical antibiotics in the food chain.	3											
There should be a global target to stop the use of antibiotics for growth promotion and infection prevention without veterinary advice	4											
Antibiotic resistance should be included as a cause of death on all national death certificates to enable collection of data on mortality.												

Q12. And, the last set of proposals is regarding surveillance and monitoring. Still on the same 0 to 10 scale.

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
National surveillance programs should be in place to monitor - infection rates, bacterial types, antibiotic use, and antibiotic resistance – in humans, animals (including fish) and water systems.	1											
International funding support should be available for surveillance in countries willing to invest in	2											

laboratory capacity, monitoring, and reporting and where systems are not yet available.												
Surveillance data should be reported nationally and regionally, and to the World Health Organization, Food and Agriculture Organisation, and the World Organisation for Animal Health.	3											
Laboratory systems for measuring antibiotic resistance in bacteria for humans and animals should be integrated with shared One Health resources.	4											
Data from point of care rapid diagnostic tests should be incorporated into routine national surveillance reporting.	5											

Q13. Now, here are some questions some people may disagree or agree with. On a 0 to 10 scale, where 0 is strongly disagree, 10 is strongly agree, and 5 is in the middle, how strongly would you disagree or agree with each of the following?

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Oppose						10 = Strongly Support				
Antibiotic resistance is one of the biggest problems the world faces	1											
Medical experts will solve the problem of antibiotic resistance before it becomes too serious	2											
The use of antibiotics in livestock and crops cannot affect me directly	3											
Increased access to antibiotics will save many lives	4											
Allowing informal sales of antibiotics leads to improper use of these medicines	5											
Incorporating point of care rapid diagnostic tests will better inform patients whether they need an antibiotic	6											
Incorporating point of care rapid diagnostic tests will reduce the use of inappropriate antibiotics	7											
Incorporating point of care rapid diagnostic test data will give a better understanding of infection rates	8											
Increased understanding of the potential negative effects of antibiotics may invoke fear of	9											

taking one when actually needed thereby potentially causing harm												
Cost of increasing infection prevention on farms could increase food costs so farmers are incentivized to use over the counter antibiotics in feed and water as cheap alternatives	10											
Penalising pharmaceutical companies for not reaching environmental targets may dissuade companies from the manufacture of antibiotics in favour of other drugs with less environmental impact	11											
The investment needed to perform necessary surveillance and reporting of over-the-counter sales from informal sellers is too large to be justified	12											

Q14. Here are some things that people find **more or less important** for themselves or society to have. On a 0 to 10 scale, where 0 is of no importance at all, 10 is extremely important, and 5 is in the middle, how important or unimportant would you say each of the following is to you?

		0	1	2	3	4	5	6	7	8	9	10
		0 = Not Important at all						10 = Extremely Important				
Everyone should have equal opportunities.	1											
The government should do what the people want.	2											
Every person should have their rights and freedoms respected.	3											
People should be able to say what they want.	4											
Making sure people follow the rules	5											
Citizens in all countries should have equal access to the medicines, diagnostics and vaccines they need	6											
Citizens in all countries should have equal access to clean water and safe sanitation	7											
Considering the interests of future generations	8											

Q15. On a 0 to 10 scale, where 0 is strongly disagree and 10 is strongly agree, how strongly would you disagree or agree with the following statements?

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Disagree						10 = Strongly Agree				
Public officials care a lot about what people like me think.	1											
Most public policy issues are so complicated that a person like me can't really understand what's going on.	2											
People like me don't have any say about what the government does.	3											
People like me can't do much about antibiotic resistance	4											
People like me can't do much about food security	5											
I have opinions about policy issues that are worth listening to.	6											
I have the ability to create change in my community.	7											

Q16. Now we'd like you to think about the people who disagree strongly with you on the issues we've been asking you about. Please rate your agreement with the following statements about those people from 0 (strongly disagree) to 10 (strongly agree). If you don't have an opinion on a statement, feel free to select "no opinion."

		0	1	2	3	4	5	6	7	8	9	10
		0 = Strongly Disagree						10 = Strongly Agree				
They just don't know enough.	1											
They believe some things that are untrue.	2											
They're not thinking clearly.	3											
They have good reasons; there just are better ones on the other side.	4											
They're looking out for their own interests.	5											
They are receiving inaccurate information.	6											
I respect their point of view though it is different from mine.	7											
It is hopeless to reach an agreement with them.	8											
I would be willing to compromise to find a solution we both can support.	9											

Q17. On a 0 to 10 scale, where 0 is "completely distrust", 10 is "completely trust", and 5 is "neither trust nor distrust", how much would you trust the following to do the right thing?

		0	1	2	3	4	5	6	7	8	9	10
		0 = Completely Distrust					5 = Neither Trust nor Distrust	10 = Completely Trust				
The government	1											
Pharmaceutical companies	2											
Your doctors	3											
Your nurses	4											
Your pharmacists	5											
Your community	6											

Now here are some **factual questions** to which not everyone may know the right answers. Please note only one answer is correct. If you come to a question to which you don't know the answer, don't worry about it. Just select "don't know" and move on to the next one.

Q18. What does antibiotic resistance mean? Choose only **one** answer.

1	Medicine that is working against your body to make you more sick
2	A virus causing infection in your body is able to resist the action of an antibiotic
3	Antibiotic manufacturers have become resistant to producing antibiotics
4	Bacteria causing an infection are able to resist the action of an antibiotic
6	Don't know

Q19. When was the latest discovery of a new antibiotic class that has reached the market?

1	2019
2	2005
3	1999
4	1987
5	1956
6	Don't Know

Q20. Which of the following statements are TRUE about antibiotics? Choose only **one** answer.

1	Antibiotics can kill bacteria.
2	Antibiotics have side-effects such as diarrhea, nausea and vomiting.
3	Antibiotics cannot kill viruses.

4	People can be allergic to antibiotics.
	All of the above
6	Don't know

Q21. Which of the following is a way to prevent infection, in order to reduce antibiotic resistance?
Choose only **one** answer.

1	Antibiotics can kill bacteria.
2	Antibiotics have side-effects such as diarrhea, nausea and vomiting.
3	Antibiotics cannot kill viruses.
4	People can be allergic to antibiotics.
	All of the above
6	Don't know

Q22. What does the term 'superbug' mean?

1	One of the largest bugs, in size, on earth that causes the worse colds
2	A bug with 'super' abilities to eat large amount of bacteria to cure illnesses
3	The bacteria that can be killed by any antibiotic
4	The most resistant types of bacteria almost always found in hospitals
5	Don't know

Q23. Which of the following statements is a fact about antibiotic resistance?

1	More children under the age of 5 years die from lack of access to an antibiotic than die of antibiotic resistant bacterial infection
2	Antibiotic use has been associated with obesity later in life
3	Antibiotic use has been associated with developing inflammatory bowel disease (ulcerative colitis and Crohn's Disease) in later life
4	Antibiotic misuse in humans is a major cause of increasing antibiotic resistance
5	All of the above
6	Don't Know

Now, just a **few more questions** about you.

Q24. Have you ever taken an antibiotic?

1	Yes	CONTINUE
2	No	SKIP TO Q29.

Q25. When did you last take an antibiotic?

1	In the last month
2	In the last 6 months
3	In the last year
4	More than a year ago

5	Can't remember
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Q26. how many different periods of treatment of antibiotics have you taken in the last year?

1	1-2
2	3-4
3	5-6
4	7-8
5	More than 8
6	Can't remember

Q27. How did you get your last period of treatment of antibiotics for your infection?

1	From the pharmacist with a prescription from a doctor
2	From the pharmacist without a prescription from a doctor
3	From a nurse who prescribed the antibiotic in a clinic
4	From a drug shop without a prescription
5	From a market seller without a prescription
6	From one of my friends without a prescription
7	From one of my family without a prescription
8	From a veterinarian
9	Can't remember

Q28. from whom do you normally get your antibiotics?

1	From the pharmacist with a prescription from a doctor
2	From the pharmacist without a prescription from a doctor
3	From a nurse who prescribed the antibiotic in a clinic
4	From a drug shop without a prescription
5	From a market seller without a prescription
6	From one of my friends without a prescription
7	From one of my family without a prescription
8	From a veterinarian

Q29. If you are a farmer or someone who keeps animal(s) for food production at your home, how many periods of treatment of antibiotics have you given your animal(s) in the last 6 months?

1	0
2	1-2
3	3-4
4	5-6
5	7-8
6	More than 8

Are you a farmer or someone who keeps animal(s) for food production at your home?

Yes	CONTINUE
No	SKIP TO Q32

Q30. If you are a farmer or someone who keeps animals for food production at your home, where did you access the last antibiotic(s) you gave your animal(s)

1	From a veterinarian
2	From a veterinarian's assistant
3	From the local co-operative already mixed in with feed without a prescription
4	From the local co-operative already mixed in with water without a prescription
5	From the local co-operative for me to mix into food or water
6	From another farmer
7	From my own stock of antibiotics that I use for myself or my family
8	I do not give my animal(s) antibiotics

Q31. If you are a farmer or someone who keeps animals for food production, which of the following statements apply. Check all that apply.

1	I give my animal(s) antibiotics to promote their growth and get them to market quicker
2	I routinely give my animal(s) antibiotics to keep them healthy, whether they are sick or not
3	I only give my animal(s) antibiotics if they are showing signs of illness
4	If my animal(s) are sick, I always consult with a veterinarian or veterinarian's assistant before my animals receive an antibiotic

Q32. How much extra, in percentage, would you be willing to pay at the shops for meat that was produced without the use of routine antibiotics (for growth promotion or to prevent infection even when the animal(s) is/are healthy)? **Please use the sliding scale to indicate what percentage you would be willing to pay more.**

INSERT 0-100 SLIDING SCALE

0%	5%	10%	20%	30%	40%	50%	60%	70%	80%	90%	100%

Q33. Which of the following antibiotics have you heard of or taken before? **[drop down menu]**

Antibiotics
Amikacin
Amoxicillin
Amoxicillin and clavulanic acid
Amoxicillin and flucloxacillin
Ampicillin and cloxacillin
Azithromycin
Cefixime
Cefixime and clavulanic acid
Cefpodoxime
Ceftriaxone
Cefuroxime
Ciprofloxacin
Clarithromycin
Clindamycin

Doxycycline
Erythromycin
Levofloxacin
Meropenem
Metronidazole
Nitrofurantoin
Ofloxacin
Secnidazole
Sparfloxacin
Ornidazole
Tinidazole
Linezolid
Vancomycin
Trimethoprim/sulfamethoxazole
None of the above

Q34. In what year were you born?

Year

Q35. Do you live in an urban or rural area?

1	City
2	Countryside (rural)

Q36. What do you live in?

1	One bedroom apartment/ flats/ Self contained
2	2-bedroom Apartments /flats
3	3-bedroom apartments/flats
4	4–5-bedroom apartment/flats
5	Duplex
6	Family House (An entire building occupied by a family)

Q37. Which of the following best describes your current employment status?

1	Employed full-time, working 30 hours or more per week
2	Employed part-time, working less than 30 hours per week
3	Self-employed/independent contractor/freelancer
4	Full-time homemaker or stay-at-home parent
5	Have been furloughed temporarily
6	Have recently been laid-off
7	Have been unemployed for 3 months or more
8	Retired or on disability
	Full-time student

Q38. (If employed) Which of the following best describes your employment?

1	Self employed
2	Business with fewer than 25 employees
3	Business with 25 or more employees
4	Government
5	Non-profit organization
6	Other
7	(If Other) What is that? (Please specify)

Q39. Which of the following best describes your household's annual income?

1	Less than #200,000
2	#200,001- #500,000
3	#500,001-#1000,000
4	#1000,001, - #2000,000
5	#2000,001-#5000,000
6	#5000,001-#600,000
7	#6000,001- #7000,000
8	#7000,001- #10,000,000
9	#10,000,000 +
10	Refused to say.

Q40. What is your highest level of education?

1	Primary Incomplete
2	Primary Complete
3	Secondary/High School
4	Undergraduate/college/Post Secondary/ Vocational
5	BSc Degree/HND/ Diploma
6	MBA/MA/MSc/
7	Postgraduate/ PhD/DBA

Q41. Which of the following racial or ethnic group would you say best describes you: White, Black/ other?

1	Black	CONTINUE TO Q43
2	White	CHECK IF AFRICA DESCENT, ASK Q42.
3	Other- Please specify	

Q42. And what is that?

If white, Are you of African descent?

1	Black - African Descent
2	White African Descent
3	Others- Specify

Q43. What is the gender you identify with?

1	Male
2	Female
3	Non-binary
4	Prefer to self-identify Optional open end
5	Prefer not to answer

END OF PRE-SURVEY. THANK YOU

*****Below post-survey questions ONLY for participants**

Finally, some questions about the discussions you have engaged in over the past days.

On a scale of 0 to 10, where 0 is "a waste of time", 10 is "extremely valuable" and 5 is exactly in the middle, how valuable was each of the following in helping you clarify your positions on the questions discussed?

	0 = A waste of time					5 = Exactly in the middle	10 = Extremely valuable						Haven't thought much about it
The small group discussions	0	1	2	3	4	5	6	7	8	9	10		99
The briefing materials	0	1	2	3	4	5	6	7	8	9	10		99
The expert panel	0	1	2	3	4	5	6	7	8	9	10		99
The event as a whole	0	1	2	3	4	5	6	7	8	9	10		99

Now, specifically about the issue guide. On a 0 to 10 scale, where 0 is strongly disagree and 10 is strongly agree, how strongly would you agree or disagree with the following statements?

1. The briefing materials presented a range of different viewpoints
2. The briefing materials presented different viewpoints in a balanced way

On the same scale, how about the plenary session?

1. The expert panel moderators facilitated a balanced discussion among experts
2. The experts presented a range of different viewpoints
3. The experts presented different viewpoints in a balanced way

And, here are some questions about on another 0 to 10 scale, where 0 is strongly disagree and 10 is strongly agree, how strongly would you agree or disagree with each of the following statements?

	0	1	2	3	4	5	6	7	8	9	10	Don't Know
1. The deliberation platform provided the opportunity for everyone to participate in the discussion												
2. The members of my group participated relatively equally in the discussions												
3. The deliberation platform sometimes tried to influence the group's opinions												
4. The deliberation platform tried to make sure that opposing arguments were considered												
5. The important aspects of the issues were covered in the group discussions												
6. I learned a lot about people very different from me – about what they and their lives are like												
7. I understand a lot more about other people's views on this topic after the discussions												
8. I felt pressured to agree with others' ideas or arguments												
9. I have learned about more arguments that												

helped me refine my opinion on this topic												
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Table 1: Forty five proposals considered by the Intervention Group

Equity, Access and Stewardship 1. Governments in low- and middle-income countries should ensure appropriate accessed to antibiotics for all citizens including those who have difficulty in accessing the health system 2. Governments in low- and middle-income countries should increase access to antibiotics to support all farmers for their animals, including those who have little access to veterinarians 3. Governments should develop clear guidelines and support for controlled informal dispensing and monitoring of the most widely needed basic oral antibiotics without an doctor's or veterinarian's prescription 4. Registered pharmacists should be able to dispense basic oral antibiotics over the counter for some community-acquired infections without a prescription 5. Informal antibiotic sellers such as Patent And Proprietary Medicine Vendors and informal drug shops should be regulated and monitored to dispense a limited number of basic oral antibiotics for specific infections under strict conditions 6. Veterinarian assistants or allied veterinary professionals should be allowed to dispense antibiotics to farmers for a limited number of infections without a prescription 7. Governments should develop monitoring (and evaluation) mechanisms to include informal sales of antibiotics 8. Governments should report the volumes of sales from these informal sources annually to the Global Antibiotic Surveillance System, a global monitoring system set up by the World Health Organization 9. Governments should incentivise informal sellers to use only quality-assured antibiotics 10. An international initiative is needed to ensure affordable steady supply of quality-assured basic oral antibiotics in low- and middle-income countries including local manufacturing 11. An international initiative is needed to ensure availability of proven/regulated point of care diagnostic tests to support appropriate antibiotic use in all countries. 12. Governments should make currently available validated point of care diagnostic tests available at every primary health outlet 13. Point of care rapid diagnostic tests need to be available and part of care pathways, and should be reimbursed by the State of Medical Insurance Companies 14. There should be national certification standards from manufacture of antibiotics in all countries for safe waste/outflow disposal, with penalties if standards are not met.
Infection Prevention 15. National governments should prioritise infection prevention tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases 16. Everyone should have access to clean water and safe sanitation (WASH) in their community and health system. 17. Specific government t funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene

18. Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
19. An adult programme of vaccination should be created by national governments to ensure vaccination of vulnerable populations for influenza and pneumonia
20. Influenza vaccination should be incentivised for patient-facing healthcare workers
21. Hospital accreditation should be linked to infection prevention, such as using hand washing rates as a factor
22. Hospital should be mandated to publish their hand hygiene rates annually
23. Farms should ensure good infection prevention and biosecurity
24. Farming, including fish, should use vaccination
25. Farm workers should be taught to wash their hands

Food Security

26. An identifying label should accompany food that is produced without the use of routine antibiotics
27. Large-scale food suppliers – supermarkets, restaurant chains, intensive farming etc., - should be incentivised to promote the use of food produced without routine antibiotics
28. Large-scale food suppliers - supermarkets, restaurant chains, intensive farming etc., - should be regulated to promote the use of food produced without routine antibiotics
29. Governments should regulate farmers to establish infection prevention programs on farms (including vaccination) linked to an inspection program
30. Group prevention therapy with antibiotics for sick animals should be under the management of a veterinarians or allied professional
31. Governments should ban from food production systems, the use of antibiotics that are critically important for humans

Awareness and Education

32. Food produced with the routine use of antibiotics should be identified by an international standard such as a hazard-warning symbol or label
33. Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through to graduation
34. Awareness campaigns should focus on the risks and harms of taking unnecessary antibiotics on humans, food security, and the environment
35. National governments should increase funding for awareness campaigns into vaccine hesitancy and improving understanding about people's behaviours relating to vaccination

Targets

36. There should be global targets to address bacterial resistance to antibiotics, just like global targets for climate
37. There should be a global target to reduce human deaths from antibiotic resistance
38. There should be a global target to stop the use of medically critical antibiotics in the food chain
39. There should be a global target to stop the use of antibiotics for growth promotion and infections prevention without veterinary advice

40. Antibiotic resistance should be included as a cause of death on all national death certificates to enable collection of data on mortality

Surveillance

- 41. National surveillance programs should be in place to monitor infection rates, bacterial types, antibiotic use, and antibiotic resistance in humans, animals (including fish) and water systems
- 42. International funding support should be available for surveillance in countries willing to invest in laboratory capacity, monitoring, and reporting and where systems are not yet available.
- 43. Surveillance data should be reported nationally, regionally, and to the World Health Organization, Food and Agriculture Organisation, and the World Organisation for Animal Health
- 44. Laboratory systems for measuring antibiotic resistance in bacteria for humans and animals should be integrated with shared One Health resources
- 45. Data from point-of-care rapid diagnostic tests should be incorporated into routine national surveillance reporting.

Table 2: Number of participants in intervention and control groups by country

Country	Treatment Group, n	Control Group, n
Tanzania	185	206
Nigeria	203	210
India	231	160
Brazil	188	178
Colombia	275	185
Indonesia	198	200

Table 3: Attendance rate of participants in the intervention deliberation groups as a percentage of those invited

Country	Invited to Deliberation	Participated in Deliberation	Attendance % from invitation
Brazil	812	188	23%
Colombia	935	275	29%
India	700	231	33%
Indonesia	238	198	83%
Nigeria	221	203	92%
Tanzania	221	185	84%

Table 4: Demographics – Control and Intervention groups. Results are expressed in % proportion in each variable in each country

Characteristic	Group	Variable	Tanzania	Nigeria	India	Colombia	Brazil	Indonesia
Gender	Control	Male (%)	51.8	51.2	69.4	52.4	49.4	50.5
		Female (%)	48.2	48.8	30.6	47.6	50.6	49.5
	Intervention	Male (%)	47.6	51.4	67.5	53.5	48.7	50.6
		Female (%)	52.4	48.6	32.5	46.6	51.3	49.4
Age, years	Control	18-24 (%)	21.4	19.5	32.3	17.3	18.0	20.5
		25-34 (%)	31.8	27.3	33.5	29.2	25.3	32.0
		35-44 (%)	13.5	23.4	18.4	19.5	22.5	23.0
		45-54 (%)	6.8	17.6	10.1	20.0	16.3	22.0
		55+ (%)	26.6	12.2	5.7	14.1	18.0	2.5
	Intervention	18-24 (%)	17.2	20.5	32.0	16.0	16.4	18.2
		25-34 (%)	31.1	37.5	32.9	29.8	26.5	33.8
		35-44 (%)	31.1	18.0	19.5	21.1	21.2	24.8
		45-54 (%)	20.6	8.5	10.0	17.8	17.5	22.7
		55+ (%)	0.0	15.5	5.6	15.3	18.5	0.5
Education	Control	Primary or less,, (%)	25.1	0.0	7.1	5.3	37.5	0.0
		Secondary (%)	41.9	24.0	63.3	59.0	48.4	59.3
		Post-Secondary*, (%)	22.3	28.2	6.3	0.0	4.1	0.0
		Tertiary or higher, (%)	10.6	47.8	23.3	35.7	10.0	40.7
	Intervention	Primary or less, (%)	17.4	0.0	6.3	5.8	33.2	1.0
		Secondary, (%)	47.9	7.0	64.4	58.6	51.5	33.7
		Post-Secondary*, (%)	23.4	18.8	6.3	0.0	4.8	0.0
		Tertiary or higher, (%)	11.3	74.2	23.1	35.6	10.6	65.3

Urbanicity	Control	Urban or Suburban	79.0	99.6	77.0	61.4	75.5	95.4
		Town, rural or other	21.0	0.4	23.0	38.6	24.5	4.6
	Intervention	Urban or Suburban	95.9	97.8	88.7	63.6	69.2	95.0
		Town, rural or other	4.1	2.2	11.3	36.4	30.9	5.0

*Post-secondary = post secondary non-tertiary or vocational education

Table 5: Top ten proposals after deliberation by topic and country

Topic	Tanzania	Nigeria	India	Colombia	Brazil	Indonesia
Infection Prevention	2	2	6	5	7	7
Awareness & Education	1	1	3	2	2	2
Surveillance & Monitoring	3	3	0	0	0	0
Targets for Antibiotics	1	1	0	1	0	0
Stewardship	0	0	1	1	0	1
Access to antibiotics	3	2	0	0	0	0
Food Security	0	1	0	1	1	0

Table 6: Top 10 proposals for Tanzania

Rank	Topic	Proposal
1	Target for antibiotics	There should be a global target to stop the use of medically critical antibiotics in the food chain.
2	Access to antibiotics	Governments in low- and middle-income countries should increase access to antibiotics to support all farmers for their animals, including those who have little access to veterinarians.
3	Access to antibiotics	Governments should develop clear guidelines and support for controlled informal dispensing and monitoring the most widely needed basic oral antibiotics without a doctor's or veterinarian's prescription
4	Surveillance and monitoring	National surveillance programs should be in place to monitor infection rates, bacterial types, antibiotic use, and antibiotic resistance in humans, animals (including fish) and water systems
5	Infection Prevention	Specific government funding should go to all healthcare facilities to ensure that all have access to clean water for hand hygiene
6	Infection Prevention	Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
7	Access to antibiotics	Informal antibiotic sellers such as Patent and Proprietary Medicine Vendors and informal drug shops should be regulated and monitored to dispense a limited number of basic oral antibiotics for specific infections under strict guidelines but still without a prescription
8	Surveillance and monitoring	Data from point of care rapid diagnostic tests should be incorporated into routine national surveillance reporting
9	Awareness and education	Awareness campaigns should focus on the risks and harms of taking unnecessary antibiotics on humans, food security, and the environment
10	Surveillance and monitoring	Laboratory systems for measuring antibiotic resistance in bacteria for humans and animals should be integrated with shared One Health resources

Table 7: Top 10 proposals for Nigeria

Rank	Topic	Proposal
1	Awareness and education	Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through graduation
2	Targets for antibiotics	There should be a global target to reduce human deaths from antibiotic resistance
3	Surveillance and monitoring	Data from point of care rapid diagnostic tests should be incorporated into routine national surveillance reporting
4	Infection Prevention	Farms should ensure good infection prevention and biosecurity
5	Surveillance and monitoring	National surveillance programs should be in place to monitor infection rates, bacterial types, antibiotic use, and antibiotic resistance in humans, animals (including fish) and water systems
6	Access to antibiotics	Informal antibiotic sellers such as Patent and Proprietary Medicine Vendors and informal drug shops should be regulated and monitored to dispense a limited number of basic oral antibiotics for specific infections under strict guidelines but still without a prescription
7	Infection Prevention	Hospitals should be mandated to publish their hand hygiene rates annually
8	Food security	Large-scale food producers – supermarkets, restaurant chains, intensive farming etc., - should be incentivized to promote the use of food produced without routine antibiotics.
9	Access to antibiotics	Governments should develop monitoring and evaluation mechanisms to include the informal sales of antibiotics
10	Surveillance and monitoring	Surveillance data should be reported nationally, regionally, and to the World health Organization, Food and Agriculture Organisation, and the World Organisation for Animal Health

Table 8: Top 10 proposals for India

Rank	Topic	Proposal
1	Infection Prevention	Everyone should have access to clean water and safe sanitation (WASH) in their community and health system
2	Awareness and education	Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through graduation
3	Infection Prevention	National governments should prioritise infection preventions tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases
4	Infection Prevention	Specific government funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene
5	Infection Prevention	Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
6	Awareness and education	Awareness campaigns should focus on the risks and harms of taking unnecessary antibiotics on humans, food security, and the environment
7	Awareness and education	National governments should increase funding for awareness campaigns into vaccine hesitancy and improving understanding about people's behaviours relating to vaccination
8	Infection Prevention	Farms should ensure good infection prevention and biosecurity
9	Infection Prevention	Hospital accreditation should be linked to infection prevention, such as using hand washing rates as a factor
10	Making best use of antibiotics	There should be national certification standards from the manufacture of antibiotics in all countries for safe waste/outflow disposal, with penalties if standards are not met.

Table 9: Top 10 proposals for Colombia

Rank	Topic	Proposal
1	Infection Prevention	Everyone should have access to clean water and safe sanitation (WASH) in their community and health system
2	Infection Prevention	National governments should prioritise infection preventions tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases
3	Infection Prevention	Specific government funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene
4	Awareness and education	Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through graduation
5	Infection Prevention	Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
6	Awareness and education	Awareness campaigns should focus on the risks and harms of taking unnecessary antibiotics on humans, food security, and the environment
7	Infection Prevention	Farm workers should be taught hand hygiene
8	Making best use of antibiotics	There should be national certification standards from the manufacture of antibiotics in call countries for safe waste/outflow disposal, with penalties if standards are not met.
9	Targets for antibiotics	There should be a global target to reduce human deaths from antibiotic resistance
10	Food security	Group prevention therapy with antibiotics for sick animals should be under the management of a veterinarian or allied professional

Table 10: Top 10 proposals for Brazil

Rank	Topic	Proposal
1	Infection Prevention	Farm workers should be taught hand hygiene
2	Infection Prevention	Everyone should have access to clean water and safe sanitation (WASH) in their community and health system
3	Infection Prevention	Farms should ensure good infection prevention and biosecurity
4	Infection Prevention	National governments should prioritise infection preventions tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases
5	Infection Prevention	Influenza vaccination should be prioritised for patient-facing healthcare workers
6	Infection Prevention	Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
7	Infection Prevention	An adult programme of vaccination should be created by national governments to ensure vaccination of vulnerable populations for influenza and pneumonia
8	Awareness and education	National governments should increase funding for awareness campaigns into vaccine hesitancy and improving understanding about people's behaviours relating to vaccination
9	Awareness and education	Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through graduation
10	Food security	Group prevention therapy with antibiotics for sick animals should be under the management of a veterinarian or allied professional

Table 11: Top 10 proposals for Indonesia

Rank	Topic	Proposal
1	Infection Prevention	Farm workers should be taught hand hygiene
2	Infection Prevention	National governments should prioritise infection preventions tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases
3	Infection Prevention	Everyone should have access to clean water and safe sanitation (WASH) in their community and health system
4	Infection Prevention	Specific government funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene
5	Making best use of antibiotics	There should be national certification standards from the manufacture of antibiotics in call countries for safe waste/outflow disposal, with penalties if standards are not met.
6	Infection Prevention	Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage to capture all children
7	Infection Prevention	Hospitals should be mandated to publish their hand hygiene rates annually
8	Awareness and education	Food produced with the routine use of antibiotics should be identified by an international standard, such as a hazard-warning symbol or label
9	Infection Prevention	Farms should ensure good infection prevention and biosecurity
10	Awareness and education	Hand hygiene and infection preventions should be part of the standard curriculum in schools from the start of schooling through graduation

Table 12: Equity Access and Stewardship - mean ratings for each proposal

Proposal	Measure	Pre								Post								Difference							
		All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR			
Governments in low- and middle-income countries should ensure appropriate access to antibiotics for all citizens including those who have difficulty in accessing the health system.	Mean	8.51	7.77	8.04	8.21	8.69	8.87	9.37	8.76	8.81	8.40	8.25	8.84	9.09	9.06	0.25***	1.04***	0.36**	0.05	0.15	0.22*	-0.31			
	Oppose	2.6%	6.9%	1.7%	1.4%	2.3%	3.0%	0.3%	0.7%	0.0%	0.0%	2.2%	0.6%	0.0%	1.6%	-1.9%	-6.9%	-1.7%	0.8%	-1.7%	-3.0%	1.4%			
	In the middle	6.2%	17.9%	1.6%	9.9%	4.8%	2.0%	3.6%	3.2%	3.5%	1.6%	7.5%	1.4%	2.1%	3.9%	-3.0%	-14.4%	-0.1%	-2.4%	-3.4%	0.1%	0.3%			
	Support	88.3%	75.2%	96.7%	84.9%	89.5%	87.6%	95.0%	94.9%	96.5%	98.4%	88.6%	96.8%	95.4%	93.3%	6.6%	21.3%	1.7%	3.7%	7.3%	7.8%	-1.7%			
	DK/NA	3.0%	0.0%	0.0%	3.8%	3.4%	7.4%	1.1%	1.2%	0.0%	0.0%	1.8%	1.2%	2.5%	1.2%	-1.8%	0.0%	0.0%	-2.1%	-2.2%	-4.9%	0.1%			
Governments in low- and middle-income countries should increase access to antibiotics to support all farmers for their animals, including those who have little access to veterinarians.	Mean	8.25	7.25	7.84	8.04	8.65	8.61	8.95	8.34	8.88	8.12	8.12	8.63	8.25	8.00	0.09	1.63***	0.27	0.08	-0.02	-0.37**	-0.95***			
	Oppose	4.8%	19.6%	3.5%	2.9%	1.7%	1.9%	1.9%	3.9%	0.8%	1.6%	3.2%	3.5%	6.1%	7.2%	-0.9%	-18.8%	-1.8%	0.3%	1.9%	4.2%	5.3%			
	In the middle	6.3%	11.6%	3.6%	9.7%	4.1%	7.3%	1.8%	5.8%	4.5%	3.6%	8.6%	2.0%	6.3%	10.4%	-0.5%	-7.1%	0.0%	-1.2%	-2.0%	-1.0%	8.7%			
	Support	85.4%	68.8%	93.0%	84.4%	92.4%	82.3%	90.5%	88.9%	94.7%	94.8%	86.5%	92.6%	84.4%	81.7%	3.5%	25.9%	1.8%	2.1%	0.2%	2.1%	-8.8%			
	DK/NA	3.5%	0.0%	0.0%	2.9%	1.8%	8.6%	5.9%	1.4%	0.0%	0.0%	1.8%	1.8%	3.3%	0.7%	-2.1%	0.0%	0.0%	-1.2%	0.0%	-5.3%	-5.2%			
Governments should develop clear guidelines and support for controlled informal dispensing and monitoring of the most widely needed basic oral antibiotics without a doctor's or veterinarian's prescription.	Mean	7.57	7.67	7.99	7.63	7.77	7.54	6.61	7.92	8.87	8.27	7.89	8.54	7.76	5.90	0.36***	1.20***	0.28	0.25	0.77***	0.22	-0.72*			
	Oppose	9.8%	13.1%	3.7%	6.6%	4.3%	11.2%	21.2%	8.2%	0.6%	1.6%	4.0%	4.3%	10.2%	29.2%	-1.6%	-12.6%	-2.1%	-2.6%	0.0%	-1.0%	8.0%			
	In the middle	9.2%	3.4%	3.9%	13.3%	12.0%	9.1%	12.6%	7.7%	1.8%	3.9%	11.5%	4.3%	10.7%	13.1%	-1.5%	-1.6%	0.0%	-1.8%	-7.7%	1.5%	0.5%			
	Support	76.5%	83.4%	92.4%	77.2%	77.8%	72.6%	56.3%	82.2%	97.6%	94.5%	83.4%	89.5%	76.0%	53.3%	5.7%	14.3%	2.1%	6.2%	11.7%	3.4%	-3.1%			
	DK/NA	4.5%	0.1%	0.0%	2.9%	5.9%	7.1%	9.8%	1.9%	0.0%	0.0%	1.1%	2.0%	3.2%	4.4%	-2.7%	-0.1%	0.0%	-1.8%	-4.0%	-3.9%	-5.4%			
Registered pharmacists should be able to dispense basic oral antibiotics over the counter for some community-acquired infections without a prescription.	Mean	6.84	5.44	7.58	7.20	7.32	7.01	6.19	6.81	7.59	7.30	7.48	7.30	5.93	5.39	-0.03	2.15***	-0.28	0.28	-0.02	-1.08***	-0.80**			
	Oppose	17.3%	34.2%	5.8%	9.9%	14.3%	16.0%	26.4%	17.0%	2.2%	7.2%	7.8%	13.9%	28.8%	37.6%	-0.3%	-31.9%	1.4%	-2.1%	-0.5%	12.8%	11.3%			
	In the middle	12.4%	24.1%	7.0%	14.4%	8.0%	11.3%	11.5%	13.6%	25.3%	11.9%	9.5%	10.4%	12.5%	13.6%	1.2%	1.2%	4.9%	-4.9%	2.4%	1.3%	2.1%			
	Support	67.0%	41.3%	87.2%	72.1%	72.2%	67.9%	57.1%	67.9%	72.5%	80.8%	81.5%	74.1%	54.6%	47.3%	0.9%	31.2%	-6.3%	9.4%	1.9%	-13.3%	-9.7%			
	DK/NA	3.4%	0.5%	0.0%	3.5%	5.5%	4.8%	5.0%	1.6%	0.0%	0.0%	1.1%	1.6%	4.1%	1.4%	-1.8%	-0.5%	0.0%	-2.4%	-3.8%	-0.8%	-3.6%			
Informal antibiotic sellers such as Patent and Proprietary Medicine Venders and informal drug shops should be regulated and monitored to dispense a limited number of basic oral antibiotics for specific infections under strict guidelines but still without a prescription.	Mean	7.44	7.61	7.74	6.90	7.59	7.66	6.97	7.54	7.65	7.63	7.28	7.58	8.14	6.62	0.10	0.04	-0.10	0.38*	-0.01	0.48*	-0.35			
	Oppose	9.0%	9.1%	4.3%	9.5%	7.3%	8.3%	16.5%	8.2%	2.2%	5.9%	7.7%	7.4%	5.8%	21.2%	-0.8%	-6.9%	1.6%	-1.8%	0.1%	-2.4%	4.7%			
	In the middle	11.5%	11.2%	5.3%	20.2%	9.2%	9.2%	15.7%	12.9%	21.0%	7.9%	14.1%	12.5%	9.7%	14.1%	1.3%	9.9%	2.6%	-6.1%	3.3%	0.5%	-1.7%			
	Support	74.5%	79.8%	90.0%	65.8%	76.8%	75.3%	58.3%	76.8%	76.8%	86.3%	77.1%	78.0%	80.9%	59.0%	2.3%	-3.0%	-3.7%	11.2%	1.1%	5.6%	0.7%			
	DK/NA	4.9%	0.0%	0.4%	4.4%	6.7%	7.2%	9.5%	2.2%	0.0%	0.0%	1.1%	2.2%	3.5%	5.7%	-2.8%	0.0%	-0.4%	-3.3%	-4.6%	-3.8%	-3.7%			
Veterinarian assistants or allied veterinary professionals should be allowed to dispense antibiotics to farmers for a limited number of infections in livestock without a prescription.	Mean	6.99	5.67	7.26	7.25	7.64	7.28	6.63	7.25	7.72	7.29	7.57	7.39	7.67	5.58	0.25**	2.05***	0.03	0.32	-0.25	0.39	-1.05***			
	Oppose	13.1%	31.0%	7.3%	8.5%	7.5%	9.6%	19.0%	10.8%	3.6%	7.9%	5.6%	11.1%	7.7%	30.3%	-2.3%	-27.3%	0.6%	-2.9%	3.6%	-1.9%	11.3%			
	In the middle	12.0%	10.5%	8.8%	16.7%	8.9%	12.6%	15.0%	10.6%	14.7%	11.1%	11.1%	6.1%	7.7%	15.1%	-1.4%	4.2%	2.3%	-5.6%	-2.8%	-4.8%	0.1%			
	Support	69.0%	58.5%	82.1%	70.3%	76.5%	65.4%	59.7%	75.6%	81.7%	81.0%	81.3%	78.4%	77.9%	51.0%	6.6%	23.1%	-1.1%	11.0%	1.9%	12.5%	-8.7%			
	DK/NA	5.9%	0.0%	1.8%	4.6%	7.2%	12.4%	6.4%	3.1%	0.0%	0.0%	2.1%	4.5%	6.6%	3.6%	-2.8%	0.0%	-1.8%	-2.5%	-2.7%	-5.8%	-2.8%			
Governments should develop monitoring (and evaluation) mechanisms to include informal sales of antibiotics.	Mean	8.05	7.58	7.98	8.02	8.07	8.49	7.98	8.51	8.68	8.44	8.09	8.93	9.16	7.26	0.45***	1.09***	0.46***	0.07	0.87***	0.67***	-0.73*			
	Oppose	4.9%	7.7%	1.4%	4.7%	4.8%	4.2%	7.1%	4.2%	2.7%	1.1%	2.5%	2.4%	0.4%	18.7%	-0.6%	-5.0%	-0.3%	-2.2%	-2.4%	-3.8%	11.6%			
	In the middle	8.6%	18.2%	6.4%	10.8%	6.6%	5.0%	6.7%	4.1%	2.3%	2.6%	8.8%	2.1%	0.6%	9.9%	-4.5%	-15.9%	-3.8%	-2.0%	-4.6%	-4.5%	3.1%			
	Support	83.9%	72.1%	92.0%	82.0%	87.3%	89.3%	76.7%	90.6%	95.0%	96.3%	87.6%	95.0%	97.3%	68.4%	6.7%	22.8%	4.3%	5.5%	7.6%	8.0%	-8.4%			
	DK/NA	2.6%	1.9%	0.3%	2.5%	1.3%	1.5%	9.5%	1.1%	0.0%	0.0%	1.1%	0.6%	1.7%	3.1%	-1.5%	-1.9%	-0.3%	-1.3%	-0.7%	0.3%	-6.4%			

Table 12 (continued)

Proposal		Pre								Post								Difference							
		Measure	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR		
Governments should report the volumes of sales from these informal sources annually to the Global Antibiotic Surveillance System (a global monitoring system set up by the World Health Organization).	Mean	8.13	7.45	8.01	7.80	8.53	8.25	8.66	8.54	8.57	8.34	8.06	8.91	8.97	8.19	0.42***	1.13***	0.33*	0.25*	0.38**	0.72***	-0.47*			
	Oppose	3.0%	5.3%	2.2%	4.6%	0.7%	4.1%	0.9%	1.5%	1.1%	0.3%	1.3%	0.8%	1.3%	4.8%	-1.4%	-4.2%	-1.9%	-3.2%	0.0%	-2.8%	3.9%			
	In the middle	7.8%	15.7%	4.5%	12.4%	4.6%	7.2%	4.0%	5.2%	4.4%	3.7%	9.9%	2.3%	2.6%	9.9%	-2.7%	-11.3%	-0.8%	-2.5%	-2.3%	-4.6%	5.9%			
	Support	84.0%	77.0%	93.4%	79.0%	91.4%	79.8%	83.3%	91.5%	94.5%	95.4%	87.1%	96.1%	91.7%	83.2%	7.5%	17.5%	2.1%	8.1%	4.7%	11.9%	0.0%			
	DK/NA	5.2%	2.0%	0.0%	4.1%	3.3%	8.9%	11.8%	1.8%	0.0%	0.6%	1.7%	0.8%	4.4%	2.1%	-3.4%	-2.0%	0.6%	-2.4%	-2.4%	-4.5%	-9.8%			
Governments should incentivise informal sellers to use only quality-assured antibiotics.	Mean	8.02	7.54	8.19	6.51	8.43	8.63	8.55	8.45	8.85	8.31	7.36	8.62	8.86	8.56	0.43***	1.31***	0.12	0.85***	0.20	0.23	0.01			
	Oppose	7.8%	11.3%	4.1%	17.4%	2.5%	6.7%	6.3%	4.4%	2.8%	0.2%	8.1%	4.6%	4.5%	6.0%	-3.4%	-8.6%	-4.0%	-9.3%	2.1%	-2.2%	-0.3%			
	In the middle	8.4%	16.1%	3.5%	20.6%	4.6%	2.3%	7.1%	5.4%	0.9%	1.4%	14.1%	6.3%	3.5%	6.5%	-3.0%	-15.3%	-2.1%	-6.5%	1.7%	1.2%	-0.5%			
	Support	81.0%	72.5%	92.4%	59.0%	89.7%	85.3%	83.1%	88.9%	96.4%	98.1%	75.8%	89.0%	88.9%	85.7%	8.0%	23.8%	5.7%	16.8%	-0.7%	3.7%	2.6%			
	DK/NA	2.8%	0.0%	0.0%	3.0%	3.3%	5.8%	3.5%	1.3%	0.0%	0.3%	2.0%	0.2%	3.1%	1.8%	-1.5%	0.0%	0.3%	-1.0%	-3.1%	-2.7%	-1.8%			
An international initiative is needed to ensure affordable steady supply of quality-assured basic oral antibiotics in the low- and middle-income countries including local manufacturing.	Mean	8.12	7.52	7.81	7.87	8.41	8.58	8.32	8.58	8.57	8.30	8.09	8.85	9.02	8.49	0.47***	1.05***	0.49***	0.22	0.43***	0.44***	0.18			
	Oppose	4.7%	17.5%	1.1%	3.4%	2.9%	2.7%	3.0%	1.6%	1.1%	0.8%	1.7%	1.4%	1.3%	3.6%	-3.1%	-16.4%	-0.3%	-1.7%	-1.5%	-1.4%	0.5%			
	In the middle	6.8%	10.9%	3.3%	11.6%	5.3%	3.2%	8.8%	4.6%	2.8%	3.0%	9.2%	3.0%	2.5%	8.0%	-2.3%	-8.1%	-0.3%	-2.5%	-2.4%	-0.7%	-0.7%			
	Support	84.9%	71.6%	94.9%	82.7%	89.5%	86.9%	81.1%	92.4%	96.0%	96.2%	88.0%	94.5%	91.7%	88.2%	7.5%	24.4%	1.3%	5.3%	5.0%	4.8%	7.1%			
	DK/NA	3.5%	0.0%	0.6%	2.3%	2.4%	7.2%	7.1%	1.4%	0.0%	0.0%	1.1%	1.2%	4.4%	0.2%	-2.1%	0.0%	-0.6%	-1.2%	-1.2%	-2.8%	-6.9%			
An international initiative is needed to ensure availability of proven/regulated point of care diagnostic tests to support appropriate antibiotic use in all countries.	Mean	8.21	8.14	7.89	7.77	8.32	8.57	8.46	8.65	8.73	8.39	8.11	8.90	8.91	8.81	0.45***	0.60***	0.50***	0.33**	0.58***	0.34**	0.35			
	Oppose	3.1%	4.9%	0.8%	4.2%	3.7%	2.3%	2.9%	2.1%	0.9%	3.6%	1.4%	0.9%	3.3%	1.9%	-1.0%	-4.0%	2.8%	-2.8%	-2.7%	1.0%	-1.0%			
	In the middle	5.6%	1.4%	7.7%	10.4%	2.3%	5.2%	7.1%	4.4%	2.0%	1.8%	8.8%	1.8%	3.5%	9.0%	-1.3%	0.6%	-5.9%	-1.6%	-0.4%	-1.7%	1.9%			
	Support	88.1%	93.7%	90.9%	83.1%	91.3%	87.1%	82.5%	92.3%	96.6%	94.6%	88.7%	95.4%	90.5%	88.1%	4.2%	2.9%	3.7%	5.6%	4.1%	3.4%	5.6%			
	DK/NA	3.2%	0.0%	0.6%	2.4%	2.8%	5.4%	7.4%	1.3%	0.5%	0.0%	1.1%	1.9%	2.7%	0.9%	-1.9%	0.5%	-0.6%	-1.2%	-0.9%	-2.7%	-6.6%			
Governments should make currently available validated point of care, rapid diagnostic tests available at every primary health outlet.	Mean	8.48	7.84	8.22	8.23	8.48	8.86	9.15	8.71	8.43	8.62	8.29	8.77	9.00	9.08	0.23***	0.59***	0.39***	0.07	0.29*	0.14	-0.08			
	Oppose	2.1%	6.7%	0.7%	3.0%	2.1%	0.5%	0.6%	1.7%	2.4%	0.4%	0.5%	3.5%	1.8%	1.2%	-0.4%	-4.3%	-0.2%	-2.5%	1.4%	1.3%	0.6%			
	In the middle	6.9%	16.6%	4.3%	8.2%	8.0%	3.6%	2.6%	3.1%	2.2%	1.0%	8.4%	1.4%	1.7%	4.6%	-3.8%	-14.4%	-3.2%	0.3%	-6.6%	-2.0%	2.1%			
	Support	88.9%	76.7%	95.1%	87.0%	87.9%	89.6%	96.0%	94.1%	95.4%	98.5%	89.6%	93.8%	94.0%	93.1%	5.2%	18.7%	3.4%	2.6%	5.8%	4.4%	-2.9%			
	DK/NA	2.1%	0.0%	0.0%	1.8%	2.0%	6.2%	0.9%	1.2%	0.1%	0.0%	1.5%	1.4%	2.6%	1.1%	-0.9%	0.1%	0.0%	-0.4%	-0.6%	-3.7%	0.2%			
Point of care rapid diagnostic tests need to be available and a part of care pathways, and should be reimbursed by the State or Medical Insurance Companies.	Mean	8.24	7.72	8.03	7.99	8.24	8.52	8.93	8.63	8.50	8.37	8.24	8.80	8.97	8.82	0.39***	0.79***	0.34**	0.25	0.56***	0.46**	-0.12			
	Oppose	4.6%	16.1%	1.7%	4.4%	2.3%	2.2%	3.4%	1.4%	3.3%	0.6%	1.0%	2.8%	0.6%	0.6%	-3.2%	-12.8%	-1.0%	-3.4%	0.5%	-1.6%	-2.8%			
	In the middle	5.4%	8.1%	2.4%	7.5%	5.4%	5.7%	3.5%	4.5%	1.2%	3.1%	9.5%	1.3%	3.9%	9.0%	-0.9%	-6.9%	0.8%	1.9%	-4.1%	-1.7%	5.5%			
	Support	85.7%	75.8%	96.0%	85.6%	85.8%	83.1%	88.3%	91.6%	95.5%	96.3%	88.4%	94.5%	87.9%	87.7%	5.8%	19.7%	0.3%	2.8%	8.7%	4.9%	-0.6%			
	DK/NA	4.2%	0.0%	0.0%	2.5%	6.5%	9.1%	4.8%	2.5%	0.0%	0.0%	1.1%	1.4%	7.6%	2.7%	-1.8%	0.0%	0.0%	-1.3%	-5.1%	-1.5%	-2.1%			
There should be national certification standards from the manufacture of antibiotics in all countries for safe waste/outflow disposal, with penalties if standards are not met.	Mean	8.57	7.56	8.30	8.32	8.73	9.10	9.21	8.91	8.62	8.53	8.46	9.03	9.33	9.37	0.34***	1.06***	0.23	0.14	0.30*	0.23**	0.16			
	Oppose	3.6%	17.1%	0.6%	3.2%	1.5%	0.4%	1.2%	0.8%	1.5%	0.0%	0.4%	2.3%	0.4%	0.2%	-2.7%	-15.6%	-0.6%	-2.8%	0.8%	0.0%	-1.0%			
	In the middle	4.7%	7.8%	5.1%	7.0%	5.4%	2.7%	1.1%	3.0%	5.8%	0.6%	7.0%	2.5%	0.5%	3.0%	-1.7%	-2.0%	-4.5%	0.0%	-2.9%	-2.2%	1.9%			
	Support	88.6%	73.9%	94.3%	87.9%	88.4%	91.1%	94.4%	95.3%	92.7%	99.4%	91.4%	94.1%	97.0%	96.3%	6.6%	18.9%	5.1%	3.5%	5.7%	5.9%	1.9%			
	DK/NA	3.1%	1.3%	0.0%	1.8%	4.7%	5.8%	3.3%	0.9%	0.0%	0.0%	1.1%	1.1%	2.1%	0.4%	-2.2%	-1.3%	0.0%	-0.7%	-3.6%	-3.7%	-2.9%			

Using Likert score, percentage levels of support, opposition, those ‘in the middle’ and those responding ‘Don’t Know or Non-Applicable’ (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by *p<0.05, **p<0.01, ***p<0.001 using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 13: Infection Prevention- mean ratings for each proposal

Proposal	Pre																				Post								Difference							
	Measure	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR														
National governments should prioritise infection prevention tools (clean water, safe sanitation and vaccination) to reduce the burden of infectious diseases.	Mean	8.88	7.94	8.31	8.65	8.96	9.54	9.58	9.12	8.58	8.83	8.67	9.26	9.59	9.58	0.24***	0.64***	0.53***	0.02	0.30**	0.05	0.01														
	Oppose	3.9%	19.3%	2.0%	2.2%	1.6%	0.4%	0.6%	0.3%	1.4%	0.2%	0.4%	0.0%	0.0%	0.2%	-3.5%	-17.9%	-1.9%	-1.7%	-1.6%	-0.4%	-0.4%														
	In the middle	3.0%	2.5%	1.6%	6.2%	3.1%	1.8%	3.1%	2.8%	3.5%	1.0%	6.8%	2.0%	0.0%	4.6%	-0.2%	1.0%	-0.6%	0.6%	-1.1%	-1.8%	1.5%														
	Support	92.2%	76.1%	96.3%	90.2%	94.5%	96.6%	95.9%	96.4%	95.1%	98.8%	91.6%	98.0%	98.6%	94.7%	4.2%	19.0%	2.5%	1.4%	3.5%	1.9%	-1.1%														
	DK/NA	1.0%	2.2%	0.0%	1.4%	0.8%	1.2%	0.4%	0.6%	0.0%	0.0%	1.1%	0.0%	1.4%	0.4%	-0.5%	-2.2%	0.0%	-0.3%	-0.8%	0.2%	0.0%														
Everyone should have access to clean water and safe sanitation (WASH) in their community and health system.	Mean	9.08	8.09	8.52	8.75	9.24	9.79	9.75	9.27	8.84	8.95	8.57	9.48	9.79	9.73	0.19***	0.75***	0.43***	-0.18	0.24*	0.01	-0.02														
	Oppose	3.5%	17.2%	3.5%	2.2%	0.9%	0.0%	0.0%	0.3%	0.0%	0.0%	0.9%	0.9%	0.0%	0.0%	-3.2%	-17.2%	-3.5%	-1.3%	0.1%	0.0%	0.0%														
	In the middle	2.0%	1.6%	3.3%	4.8%	1.0%	0.7%	1.0%	1.7%	1.6%	0.0%	7.1%	2.0%	0.0%	0.3%	-0.2%	0.1%	-3.3%	2.3%	0.9%	-0.7%	-0.7%														
	Support	93.7%	79.9%	92.2%	91.6%	98.1%	98.2%	98.6%	97.0%	98.4%	100.0%	90.2%	97.1%	96.9%	99.7%	3.3%	18.5%	7.8%	-1.4%	-1.0%	-1.3%	1.1%														
	DK/NA	0.9%	1.3%	1.0%	1.4%	0.0%	1.1%	0.4%	0.9%	0.0%	0.0%	1.8%	0.0%	3.1%	0.0%	0.1%	-1.3%	-1.0%	0.3%	0.0%	2.0%	-0.4%														
Specific government funding should go to healthcare facilities to ensure that all have access to clean water for hand hygiene.	Mean	8.90	7.93	8.59	8.60	9.06	9.49	9.46	9.06	8.56	8.86	8.56	9.22	9.54	9.39	0.16***	0.63**	0.27*	-0.04	0.15	0.05	-0.07														
	Oppose	3.1%	16.6%	0.0%	2.7%	1.4%	0.0%	0.0%	1.1%	4.1%	0.8%	0.4%	1.9%	0.0%	0.0%	-1.9%	-12.5%	0.8%	-2.3%	0.5%	0.0%	0.0%														
	In the middle	2.7%	3.7%	1.3%	5.8%	4.1%	1.3%	0.4%	3.6%	4.3%	0.6%	6.4%	2.8%	2.4%	6.1%	0.9%	0.6%	-0.7%	0.6%	-1.3%	1.1%	5.7%														
	Support	93.5%	79.2%	98.7%	90.1%	94.0%	97.6%	99.1%	94.6%	91.5%	98.6%	91.7%	95.2%	95.8%	93.6%	1.0%	12.3%	-0.1%	1.7%	1.2%	-1.7%	-5.5%														
	DK/NA	0.7%	0.5%	0.0%	1.4%	0.5%	1.1%	0.4%	0.7%	0.1%	0.0%	1.4%	0.1%	1.8%	0.2%	0.0%	-0.4%	0.0%	0.0%	-0.4%	0.6%	-0.2%														
Immunization for WHO-recommended childhood vaccination should be scaled up towards universal/full coverage, to capture all children.	Mean	8.74	7.46	8.37	8.48	8.89	9.50	9.48	9.08	9.00	8.82	8.45	9.16	9.43	9.56	0.34***	1.54***	0.45***	-0.03	0.27*	-0.07	0.08														
	Oppose	2.7%	11.3%	2.4%	2.1%	0.7%	0.8%	0.9%	1.1%	1.6%	0.0%	1.2%	1.1%	1.8%	0.5%	-1.7%	-9.6%	-2.4%	-0.9%	0.4%	1.0%	-0.3%														
	In the middle	6.3%	23.6%	0.7%	8.0%	6.0%	1.1%	1.7%	1.4%	0.3%	1.3%	6.5%	0.0%	0.4%	0.6%	-4.9%	-23.3%	0.6%	-1.5%	-6.0%	-0.7%	-1.1%														
	Support	88.4%	64.6%	96.0%	88.0%	91.4%	91.9%	95.1%	96.5%	97.9%	98.7%	91.2%	98.5%	94.5%	98.4%	8.0%	33.3%	2.8%	3.2%	7.1%	2.7%	3.3%														
	DK/NA	2.5%	0.5%	1.0%	1.9%	1.8%	6.2%	2.4%	1.0%	0.1%	0.0%	1.1%	0.4%	3.2%	0.4%	-1.5%	-0.4%	-1.0%	-0.7%	-1.5%	-2.9%	-1.9%														
An adult programme of vaccination should be created by national governments to ensure vaccination of vulnerable populations for influenza and pneumonia.	Mean	8.52	7.55	7.98	8.04	8.81	9.20	9.22	8.84	8.45	8.57	8.30	8.98	9.16	9.46	0.33***	0.90***	0.60***	0.26*	0.17	-0.04	0.24														
	Oppose	2.9%	6.0%	1.8%	3.9%	2.2%	1.8%	2.1%	1.2%	1.3%	0.0%	0.9%	0.1%	3.9%	0.2%	-1.6%	-4.7%	-1.8%	-3.0%	-2.1%	2.1%	-1.9%														
	In the middle	5.5%	7.8%	1.5%	10.5%	5.4%	4.2%	4.1%	3.7%	8.3%	0.0%	7.2%	2.9%	2.1%	2.6%	-1.8%	0.5%	-1.5%	-3.3%	-2.5%	-2.1%	-1.5%														
	Support	89.6%	84.7%	94.2%	83.1%	90.8%	90.8%	93.2%	93.6%	90.4%	99.4%	90.2%	95.0%	90.9%	96.3%	4.0%	5.7%	5.2%	7.1%	4.2%	0.1%	3.1%														
	DK/NA	2.0%	1.4%	2.5%	2.5%	1.6%	3.1%	0.7%	1.5%	0.0%	0.6%	1.8%	2.0%	3.0%	0.9%	-0.5%	-1.4%	-1.9%	-0.7%	0.4%	-0.1%	0.2%														
Influenza vaccination should be incentivized for patient-facing healthcare workers.	Mean	8.49	7.69	7.76	8.22	8.61	8.98	9.49	8.76	8.33	8.29	8.21	8.87	9.14	9.57	0.27***	0.64**	0.53***	-0.02	0.26*	0.16	0.08														
	Oppose	3.2%	10.0%	1.2%	2.1%	1.5%	4.1%	0.6%	2.1%	1.5%	0.0%	3.0%	3.3%	3.8%	0.0%	-1.1%	-8.5%	-1.2%	0.9%	1.9%	-0.3%	-0.6%														
	In the middle	5.6%	11.0%	6.0%	10.1%	3.5%	3.2%	1.3%	5.0%	6.8%	6.6%	7.6%	2.9%	3.8%	3.5%	-0.6%	-4.2%	0.6%	-2.5%	-0.7%	0.6%	2.2%														
	Support	88.9%	78.2%	92.3%	84.6%	88.2%	91.8%	96.4%	90.8%	91.7%	93.4%	88.3%	86.1%	90.1%	96.5%	1.9%	13.5%	1.1%	3.8%	-2.2%	-1.7%	0.1%														
	DK/NA	2.4%	0.8%	0.5%	3.3%	6.8%	0.9%	1.6%	2.1%	0.0%	0.0%	1.1%	7.7%	2.3%	0.0%	-0.3%	-0.8%	-0.5%	-2.1%	1.0%	1.4%	-1.6%														
Hospital accreditation should be linked to infection prevention, such as using hand washing rates as a factor.	Mean	8.59	8.17	8.11	8.39	8.69	9.11	8.89	8.76	8.54	8.54	8.25	9.05	9.02	9.07	0.17***	0.37*	0.43***	-0.14	0.35**	-0.09	0.18														
	Oppose	2.9%	6.1%	2.1%	2.2%	2.7%	2.3%	2.1%	1.2%	1.0%	0.0%	2.7%	0.2%	1.8%	1.2%	-1.7%	-5.1%	-2.1%	0.5%	-2.5%	-0.5%	-0.9%														
	In the middle	4.3%	4.2%	2.7%	6.9%	3.5%	3.3%	5.8%	4.1%	8.0%	1.6%	6.9%	0.5%	3.7%	4.9%	-0.2%	3.8%	-1.1%	0.0%	-3.0%	0.3%	-0.9%														
	Support	90.0%	89.6%	94.2%	88.8%	89.7%	91.2%	86.0%	92.6%	91.0%	98.4%	88.8%	95.9%	91.3%	89.6%	2.5%	1.4%	4.2%	-0.1%	6.2%	0.1%	3.6%														
	DK/NA	2.8%	0.0%	1.0%	2.1%	4.1%	3.2%	6.1%	2.2%	0.0%	0.0%	1.6%	3.4%	3.2%	4.2%	-0.6%	0.0%	-1.0%	-0.5%	-0.6%	0.1%	-1.9%														

Table 13 (continued)

Proposal		Pre								Post								Difference							
		Measure	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR		
Hospitals should be mandated to publish their hand hygiene rates annually.	Mean	8.19	7.71	7.92	8.50	8.69	7.86	8.47	8.43	8.13	8.53	8.45	8.88	7.87	8.73	0.23***	0.43*	0.62***	-0.05	0.19	0.02	0.26			
	Oppose	5.9%	17.8%	1.4%	1.2%	2.4%	9.0%	4.1%	1.9%	0.6%	0.3%	0.4%	1.0%	4.9%	3.3%	-4.0%	-17.1%	-1.0%	-0.8%	-1.5%	-4.1%	-0.8%			
	In the middle	8.8%	10.4%	7.7%	6.6%	5.3%	12.9%	9.2%	7.5%	9.3%	0.6%	5.3%	2.7%	14.7%	10.7%	-1.3%	-1.2%	-7.0%	-1.2%	-2.6%	1.8%	1.5%			
	Support	81.1%	71.8%	90.7%	90.8%	88.9%	67.6%	79.7%	87.9%	90.1%	96.1%	93.1%	95.8%	72.4%	84.4%	6.8%	18.3%	5.4%	2.3%	6.9%	4.8%	4.7%			
	DK/NA	4.2%	0.0%	0.3%	1.4%	3.5%	10.5%	7.0%	2.7%	0.0%	2.9%	1.1%	0.6%	8.0%	1.6%	-1.5%	0.0%	2.6%	-0.3%	-2.9%	-2.5%	-5.4%			
Farms should ensure good infection prevention and biosecurity	Mean	8.55	7.72	8.11	8.33	8.73	9.01	9.17	8.87	8.26	8.53	8.34	9.05	9.24	9.60	0.32***	0.54*	0.42***	0.00	0.32**	0.23*	0.43***			
	Oppose	3.4%	18.0%	1.0%	0.8%	1.1%	1.2%	0.6%	0.7%	2.1%	0.0%	0.3%	1.6%	0.4%	0.0%	-2.7%	-15.9%	-1.0%	-0.5%	0.5%	-0.8%	-0.6%			
	In the middle	4.6%	7.8%	3.5%	7.6%	2.9%	4.5%	1.4%	3.8%	8.9%	3.8%	8.0%	1.0%	2.2%	0.4%	-0.7%	1.1%	0.3%	0.3%	-1.9%	-2.3%	-1.0%			
	Support	89.9%	73.4%	95.5%	87.5%	92.8%	92.1%	96.0%	94.2%	88.5%	96.2%	90.0%	96.0%	95.6%	97.8%	4.3%	15.1%	0.7%	2.5%	3.2%	3.5%	1.9%			
	DK/NA	2.1%	0.7%	0.0%	4.1%	3.2%	2.2%	2.0%	1.2%	0.5%	0.0%	1.8%	1.4%	1.8%	1.7%	-0.9%	-0.2%	0.0%	-2.4%	-1.8%	-0.4%	-0.3%			
Farming, including fish, should use vaccination	Mean	7.98	7.62	7.80	8.19	8.23	7.65	8.52	8.11	8.21	7.87	8.08	8.21	7.88	8.53	0.13	0.59**	0.08	-0.11	-0.02	0.23	0.01			
	Oppose	6.0%	12.4%	1.2%	1.2%	6.8%	9.6%	3.7%	5.1%	7.4%	3.0%	2.4%	7.3%	7.1%	2.0%	-1.0%	-5.0%	1.8%	1.1%	0.5%	-2.5%	-1.7%			
	In the middle	6.3%	7.7%	4.2%	8.1%	6.0%	6.2%	6.1%	7.7%	5.5%	3.7%	8.3%	5.6%	13.3%	8.2%	1.4%	-2.2%	-0.5%	0.2%	-0.4%	7.1%	2.1%			
	Support	77.5%	79.8%	93.2%	84.4%	79.6%	61.7%	71.8%	80.5%	87.1%	90.1%	87.1%	82.2%	65.0%	77.3%	3.0%	7.4%	-3.1%	2.7%	2.5%	3.3%	5.5%			
	DK/NA	10.2%	0.1%	1.4%	6.3%	7.6%	22.5%	18.4%	6.7%	0.0%	3.2%	2.3%	4.9%	14.6%	12.4%	-3.4%	-0.1%	1.8%	-4.0%	-2.7%	-7.8%	-6.0%			
Farm workers should be taught hand hygiene	Mean	8.80	7.98	8.35	8.71	8.89	9.21	9.49	9.03	8.40	8.86	8.68	9.02	9.33	9.76	0.23***	0.42*	0.51***	-0.03	0.13	0.12	0.27**			
	Oppose	4.2%	19.0%	0.0%	1.2%	3.7%	3.0%	0.0%	1.2%	2.0%	0.4%	0.4%	1.4%	2.3%	0.0%	-3.1%	-17.1%	0.4%	-0.8%	-2.3%	-0.7%	0.0%			
	In the middle	3.1%	4.0%	2.6%	4.8%	2.9%	3.0%	1.1%	3.9%	8.7%	0.9%	5.4%	5.5%	2.1%	1.3%	0.8%	4.7%	-1.7%	0.6%	2.6%	-0.9%	0.2%			
	Support	91.4%	76.8%	97.4%	91.0%	91.9%	92.9%	96.7%	94.0%	89.3%	97.6%	93.0%	91.7%	94.5%	98.0%	2.7%	12.5%	0.2%	2.1%	-0.1%	1.6%	1.3%			
	DK/NA	1.3%	0.2%	0.0%	3.0%	1.6%	1.1%	2.2%	1.0%	0.1%	1.0%	1.1%	1.4%	1.2%	0.7%	-0.4%	-0.2%	1.0%	-1.9%	-0.2%	0.1%	-1.4%			

Using Likert score, percentage levels of support, opposition, those ‘in the middle’ and those responding ‘Don’t Know or Non-Applicable’ (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by * $p<0.05$, ** $p<0.01$, *** $p<0.001$ using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 14: Food Security- mean ratings for each proposal

Proposal	Measure	Pre								Post								Difference							
		All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR			
An identifying label should accompany food that is produced without the use of routine antibiotics	Mean	8.17	8.02	7.73	7.39	8.19	8.83	8.67	8.58	8.50	8.26	7.87	8.42	9.19	9.08	0.41***	0.48*	0.54***	0.48***	0.23	0.36**	0.42*			
	Oppose	4.6%	6.1%	4.0%	5.7%	5.3%	3.0%	3.9%	2.9%	2.5%	1.0%	3.1%	5.9%	1.3%	3.7%	-1.7%	-3.7%	-2.9%	-2.6%	0.6%	-1.7%	-0.1%			
	In the middle	8.1%	10.0%	3.0%	16.5%	7.4%	4.6%	8.8%	4.8%	3.0%	6.8%	11.4%	3.9%	1.9%	2.8%	-3.3%	-7.0%	3.8%	-5.1%	-3.6%	-2.7%	-6.1%			
	Support	80.7%	77.6%	92.0%	71.2%	79.0%	82.9%	80.8%	90.3%	94.5%	92.1%	83.5%	85.5%	93.6%	92.3%	9.5%	16.9%	0.1%	12.3%	6.5%	10.8%	11.5%			
	DK/NA	6.6%	6.2%	1.0%	6.6%	8.2%	9.6%	6.5%	2.0%	0.0%	0.0%	2.0%	4.7%	3.2%	1.2%	-4.6%	-6.2%	-1.0%	-4.6%	-3.5%	-6.4%	-5.3%			
Large-scale food suppliers – supermarkets, restaurant chains, intensive farming etc., – should be incentivized to promote the use of food produced without routine antibiotics.	Mean	7.88	7.76	7.56	6.95	8.19	8.44	8.37	8.24	8.56	8.16	7.26	8.14	8.74	8.60	0.36***	0.80***	0.60***	0.32	-0.05	0.31	0.22			
	Oppose	5.8%	6.8%	4.9%	9.2%	6.5%	3.9%	4.2%	4.1%	3.1%	0.8%	5.3%	8.7%	3.3%	3.2%	-1.7%	-3.7%	-4.2%	-3.9%	2.1%	-0.6%	-1.0%			
	In the middle	8.6%	9.3%	6.5%	16.6%	4.9%	6.7%	8.7%	7.7%	1.9%	6.3%	15.2%	6.0%	8.5%	7.9%	-0.8%	-7.4%	-0.2%	-1.5%	1.2%	1.8%	-0.9%			
	Support	77.8%	83.7%	87.9%	68.0%	81.9%	71.8%	74.9%	84.1%	94.5%	91.0%	77.9%	82.7%	80.2%	80.4%	6.3%	10.8%	3.0%	9.9%	0.8%	8.3%	5.5%			
	DK/NA	7.9%	0.2%	0.6%	6.1%	6.6%	17.5%	12.2%	4.1%	0.5%	1.9%	1.6%	2.6%	8.0%	8.6%	-3.8%	0.3%	1.3%	-4.5%	-4.0%	-9.6%	-3.7%			
Large-scale food suppliers – supermarkets, restaurant chains, intensive farming etc., – should be regulated to promote the use of food produced without routine antibiotics.	Mean	7.81	7.46	7.69	7.76	7.95	8.37	7.48	8.11	8.54	7.97	7.89	8.16	8.61	7.35	0.29***	0.98***	0.29	0.13	0.21	0.24	-0.13			
	Oppose	7.4%	10.9%	5.1%	2.9%	9.7%	5.0%	12.3%	4.8%	1.4%	0.6%	1.2%	9.0%	3.8%	12.5%	-2.7%	-9.5%	-4.5%	-1.7%	-0.8%	-1.2%	0.3%			
	In the middle	10.0%	12.1%	6.6%	13.6%	7.1%	8.8%	13.1%	8.7%	9.1%	6.7%	12.0%	3.5%	7.9%	14.6%	-1.3%	-3.0%	0.1%	-1.6%	-3.5%	-0.9%	1.6%			
	Support	75.3%	75.3%	87.7%	79.0%	76.5%	70.7%	63.5%	82.0%	89.5%	92.7%	85.3%	83.0%	79.6%	61.8%	6.6%	14.2%	5.0%	6.3%	6.5%	8.9%	-1.8%			
	DK/NA	7.3%	1.8%	0.6%	4.5%	6.7%	15.6%	11.2%	4.6%	0.0%	0.0%	1.5%	4.5%	8.7%	11.1%	-2.7%	-1.8%	-0.6%	-3.0%	-2.2%	-6.9%	-0.1%			
Governments should regulate farmers to establish infection prevention programs on farms (including vaccination) linked to an inspection program.	Mean	8.38	7.37	7.99	8.09	8.63	8.95	9.08	8.66	8.49	8.42	8.13	8.59	9.11	9.14	0.28***	1.12***	0.43***	0.04	-0.03	0.17	0.06			
	Oppose	2.8%	9.7%	1.7%	1.2%	0.6%	2.6%	1.7%	1.1%	1.3%	0.6%	0.5%	2.6%	0.4%	1.2%	-1.7%	-8.4%	-1.1%	-0.8%	2.0%	-2.2%	-0.5%			
	In the middle	7.6%	22.6%	6.0%	8.6%	4.1%	4.1%	3.5%	3.7%	4.0%	1.5%	7.4%	2.4%	3.2%	4.5%	-3.9%	-18.6%	-4.5%	-1.2%	-1.7%	-0.9%	1.0%			
	Support	86.7%	66.2%	92.3%	87.2%	93.0%	90.1%	87.2%	93.3%	94.6%	97.8%	90.6%	93.5%	92.9%	90.1%	6.6%	28.4%	5.6%	3.4%	0.5%	2.9%	2.9%			
	DK/NA	2.9%	1.5%	0.0%	2.9%	2.3%	3.2%	7.6%	1.9%	0.1%	0.0%	1.5%	1.5%	3.5%	4.2%	-1.0%	-1.4%	0.0%	-1.4%	-0.8%	0.2%	-3.4%			
Group prevention therapy with antibiotics for sick animals should be under the management of a veterinarian or allied professional.	Mean	8.45	7.47	7.81	8.21	8.45	9.09	9.46	8.72	8.30	8.01	8.24	8.82	9.31	9.43	0.27***	0.84***	0.20	0.04	0.38***	0.23*	-0.03			
	Oppose	2.5%	9.7%	1.1%	0.8%	4.1%	0.3%	0.0%	1.5%	3.2%	2.9%	0.8%	0.4%	0.9%	1.4%	-1.0%	-6.5%	1.8%	0.0%	-3.7%	0.6%	1.4%			
	In the middle	7.3%	20.1%	8.6%	9.9%	3.6%	3.2%	1.3%	3.9%	3.8%	4.4%	8.2%	3.9%	2.3%	1.5%	-3.3%	-16.3%	-4.2%	-1.6%	0.2%	-0.9%	0.3%			
	Support	87.0%	69.2%	88.3%	86.4%	88.7%	90.5%	96.1%	92.9%	93.0%	91.8%	89.8%	91.9%	94.2%	96.6%	5.9%	23.9%	3.5%	3.4%	3.2%	3.7%	0.5%			
	DK/NA	3.2%	1.1%	2.0%	2.9%	3.5%	6.0%	2.6%	1.6%	0.0%	1.0%	1.1%	3.8%	2.6%	0.4%	-1.6%	-1.1%	-1.0%	-1.8%	0.3%	-3.4%	-2.2%			
Governments should ban from food production systems the use of antibiotics that are critically important for humans.	Mean	7.33	6.67	7.31	7.32	7.95	8.01	6.28	7.82	8.49	7.37	7.92	7.84	8.40	6.46	0.49***	1.82***	0.06	0.60***	-0.11	0.39	0.19			
	Oppose	10.2%	13.8%	9.2%	6.2%	6.4%	8.2%	19.7%	7.9%	2.6%	7.5%	1.0%	8.9%	8.0%	19.2%	-2.4%	-11.1%	-1.7%	-5.2%	2.5%	-0.3%	-0.6%			
	In the middle	12.3%	19.2%	4.6%	17.8%	9.5%	7.8%	17.8%	9.9%	6.1%	11.3%	14.9%	4.5%	6.7%	17.8%	-2.4%	-13.1%	6.7%	-3.0%	-4.9%	-1.1%	-0.1%			
	Support	69.3%	67.1%	84.7%	66.4%	74.8%	71.3%	48.6%	76.6%	91.3%	80.0%	83.0%	79.0%	78.3%	46.8%	7.3%	24.2%	-4.7%	16.6%	4.3%	7.0%	-1.8%			
	DK/NA	8.2%	0.0%	1.6%	9.6%	9.3%	12.6%	13.9%	5.6%	0.0%	1.3%	1.1%	7.5%	7.0%	16.3%	-2.5%	0.0%	-0.3%	-8.4%	-1.8%	-5.6%	2.4%			

Using Likert score, percentage levels of support, opposition, those ‘in the middle’ and those responding ‘Don’t Know or Non-Applicable’ (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by *p<0.05, **p<0.01, ***p<0.001 using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 15: Awareness and Education- mean ratings for each proposal

Proposal	Measure	Pre								Post								Difference							
		All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR			
Food produced with the routine use of antibiotics should be identified by an international standard, such as a hazard-warning symbol or label.	Mean	8.46	8.26	7.82	8.32	8.45	9.09	8.70	8.70	8.32	8.56	8.42	8.63	9.15	9.04	0.24***	0.05	0.74***	0.10	0.19	0.06	0.33			
	Oppose	2.7%	3.1%	2.6%	0.8%	3.2%	3.3%	3.0%	1.8%	3.4%	1.6%	1.3%	2.6%	0.4%	1.8%	-0.9%	0.3%	-1.0%	0.5%	-0.6%	-2.9%	-1.2%			
	In the middle	5.8%	8.6%	3.2%	9.2%	4.8%	5.5%	4.0%	4.2%	5.2%	0.9%	6.8%	5.3%	3.5%	4.1%	-1.5%	-3.4%	-2.3%	-2.4%	0.5%	-2.0%	0.1%			
	Support	88.4%	86.1%	94.2%	88.1%	88.0%	87.1%	86.9%	91.1%	91.4%	97.1%	90.8%	89.3%	88.9%	90.4%	2.8%	5.3%	2.9%	2.7%	1.3%	1.8%	3.6%			
	DK/NA	3.2%	2.2%	0.0%	1.9%	4.0%	4.1%	6.2%	2.9%	0.0%	0.4%	1.1%	2.8%	7.2%	3.7%	-0.3%	-2.2%	0.4%	-0.8%	-1.3%	3.1%	-2.5%			
	Mean	8.72	8.21	8.37	8.29	8.82	9.12	9.35	8.95	8.45	8.52	8.32	9.31	9.46	9.43	0.23***	0.24	0.14	0.03	0.49***	0.34***	0.08			
Hand hygiene and infection prevention should be part of the standard curriculum in schools from the start of schooling through to graduation.	Oppose	2.2%	8.0%	0.3%	1.7%	0.8%	1.2%	2.3%	0.6%	0.5%	0.6%	1.1%	0.4%	0.3%	0.7%	-1.6%	-7.5%	0.2%	-0.6%	-0.4%	-1.0%	-1.5%			
	In the middle	5.4%	8.3%	2.9%	8.4%	6.6%	3.8%	2.8%	3.1%	4.0%	2.7%	7.6%	0.3%	1.7%	3.5%	-2.3%	-4.2%	-0.2%	-0.8%	-6.3%	-2.1%	0.7%			
	Support	91.1%	83.7%	96.7%	88.1%	89.8%	93.5%	93.4%	95.8%	95.5%	96.7%	90.2%	99.0%	96.9%	95.5%	4.7%	11.7%	-0.1%	2.1%	9.2%	3.4%	2.1%			
	DK/NA	1.3%	0.0%	0.0%	1.8%	2.8%	1.5%	1.6%	0.5%	0.0%	0.0%	1.1%	0.3%	1.2%	0.3%	-0.8%	0.0%	0.0%	-0.7%	-2.5%	-0.3%	-1.2%			
	Mean	8.50	7.97	8.08	7.82	8.70	9.05	9.20	8.82	8.18	8.59	8.13	9.13	9.36	9.31	0.32***	0.21	0.51***	0.32*	0.43***	0.31***	0.11			
Awareness campaigns should focus on the risks and harms of taking unnecessary antibiotics on humans, food security, and the environment.	Oppose	2.7%	6.1%	3.5%	3.0%	1.4%	1.6%	1.6%	1.5%	6.8%	0.0%	1.1%	0.0%	0.3%	1.7%	-1.3%	0.7%	-3.5%	-1.9%	-1.4%	-1.3%	0.1%			
	In the middle	8.2%	18.7%	1.7%	16.3%	5.6%	3.6%	6.1%	3.2%	5.1%	2.5%	7.7%	2.1%	1.1%	1.9%	-5.0%	-13.7%	0.8%	-8.7%	-3.6%	-2.5%	-4.2%			
	Support	86.8%	75.2%	94.8%	76.5%	91.4%	90.5%	89.2%	93.9%	88.1%	96.5%	89.7%	97.9%	95.4%	94.2%	7.1%	12.9%	1.7%	13.2%	6.4%	4.8%	5.0%			
	DK/NA	2.3%	0.0%	0.0%	4.2%	1.5%	4.3%	3.1%	1.4%	0.0%	1.0%	1.5%	0.1%	3.3%	2.2%	-0.9%	0.0%	1.0%	-2.6%	-1.4%	-1.0%	-0.9%			
	Mean	8.42	7.89	8.05	7.84	8.62	8.95	8.95	8.80	8.41	8.53	7.97	9.11	9.19	9.44	0.39***	0.53***	0.48***	0.14	0.48***	0.24*	0.49***			
National governments should increase funding for awareness campaigns into vaccine hesitancy and improving understanding about people's behaviours relating to vaccination.	Oppose	2.3%	5.4%	2.0%	3.3%	1.3%	1.5%	1.0%	0.8%	1.8%	0.4%	2.0%	0.4%	0.3%	0.0%	-1.5%	-3.6%	-1.6%	-1.3%	-0.9%	-1.2%	-1.0%			
	In the middle	6.8%	7.8%	5.4%	10.8%	5.3%	5.9%	6.4%	3.0%	4.6%	1.4%	9.1%	0.9%	1.6%	1.2%	-3.9%	-3.2%	-4.0%	-1.8%	-4.4%	-4.3%	-5.2%			
	Support	88.3%	86.8%	92.6%	82.3%	89.6%	88.4%	90.0%	95.2%	93.6%	98.2%	87.1%	98.6%	95.2%	97.6%	6.8%	6.9%	5.6%	4.9%	9.0%	6.8%	7.6%			
	DK/NA	2.5%	0.0%	0.0%	3.6%	3.8%	4.2%	2.6%	1.1%	0.0%	0.0%	1.8%	0.1%	2.9%	1.2%	-1.4%	0.0%	0.0%	-1.9%	-3.6%	-1.3%	-1.4%			
	Mean	8.42	7.89	8.05	7.84	8.62	8.95	8.95	8.80	8.41	8.53	7.97	9.11	9.19	9.44	0.39***	0.53***	0.48***	0.14	0.48***	0.24*	0.49***			

Using Likert score, percentage levels of support, opposition, those 'in the middle' and those responding 'Don't Know or Non-Applicable' (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by * $p < 0.05$, ** $p < 0.01$, *** $p < 0.001$ using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 16: Targets- mean ratings for each proposal

Proposal	Measure	Pre								Post								Difference							
		All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR			
There should be global targets to address bacterial resistance to antibiotics, just like global targets for climate.	Mean	8.30	7.77	7.80	7.94	8.40	8.87	8.90	8.67	8.50	8.30	8.19	8.86	9.16	8.88	0.37***	0.73***	0.50***	0.25**	0.47***	0.29*	-0.02			
	Oppose	3.7%	17.3%	2.9%	0.8%	1.3%	1.5%	0.9%	1.7%	1.1%	0.6%	0.9%	1.1%	1.7%	4.9%	-2.0%	-16.1%	-2.2%	0.1%	-0.2%	0.2%	4.0%			
	In the middle	6.9%	5.8%	4.9%	11.0%	9.0%	5.5%	5.0%	4.4%	5.0%	3.1%	8.6%	3.2%	3.0%	4.4%	-2.5%	-0.8%	-1.8%	-2.5%	-5.8%	-2.5%	-0.7%			
	Support	85.1%	75.3%	91.9%	83.7%	87.3%	83.1%	89.1%	92.3%	93.9%	96.3%	89.4%	93.9%	90.4%	90.2%	7.2%	18.6%	4.3%	5.8%	6.6%	7.3%	1.1%			
	DK/NA	4.3%	1.7%	0.3%	4.5%	2.4%	9.9%	5.0%	1.6%	0.0%	0.0%	1.1%	1.8%	4.9%	0.5%	-2.6%	-1.7%	-0.3%	-3.4%	-0.5%	-5.0%	-4.4%			
There should be a global target to reduce human deaths from antibiotic resistance.	Mean	8.47	7.87	7.99	8.18	8.52	9.13	8.92	8.76	8.34	8.51	8.19	8.95	9.32	9.03	0.29***	0.48*	0.51***	0.02	0.43***	0.19*	0.11			
	Oppose	2.8%	8.2%	4.0%	2.0%	0.8%	2.5%	0.2%	0.8%	2.8%	0.4%	1.0%	0.6%	0.0%	0.9%	-2.0%	-5.5%	-3.5%	-1.0%	-0.2%	-2.5%	0.8%			
	In the middle	8.1%	21.0%	3.3%	7.7%	9.1%	3.0%	7.4%	3.5%	3.1%	1.5%	6.9%	3.4%	2.7%	4.0%	-4.6%	-17.9%	-1.8%	-0.8%	-5.7%	-0.3%	-3.4%			
	Support	85.2%	70.8%	92.8%	85.7%	86.7%	88.0%	84.7%	94.3%	93.3%	98.1%	90.4%	94.7%	94.6%	94.3%	9.1%	22.5%	5.3%	4.7%	8.0%	6.6%	9.7%			
	DK/NA	3.9%	0.0%	0.0%	4.6%	3.5%	6.4%	7.8%	1.3%	0.9%	0.0%	1.8%	1.4%	2.7%	0.7%	-2.5%	0.9%	0.0%	-2.9%	-2.1%	-3.8%	-7.1%			
There should be a global target to stop the use of medically critical antibiotics in the food chain.	Mean	7.86	7.28	7.37	7.93	8.25	8.56	7.46	8.36	8.54	8.06	8.07	8.89	8.89	7.31	0.51***	1.26***	0.69***	0.14	0.64***	0.33*	-0.15			
	Oppose	5.3%	3.8%	8.2%	1.8%	3.3%	4.4%	11.2%	4.1%	3.7%	1.5%	2.0%	2.1%	3.3%	12.9%	-1.2%	-0.1%	-6.6%	0.2%	-1.3%	-1.1%	1.7%			
	In the middle	12.5%	29.4%	7.8%	12.5%	9.1%	8.3%	11.8%	7.4%	4.1%	7.3%	9.7%	1.8%	6.2%	17.1%	-5.1%	-25.3%	-0.6%	-2.8%	-7.4%	-2.1%	5.3%			
	Support	74.8%	65.2%	82.4%	79.8%	85.8%	72.7%	60.3%	84.5%	92.2%	91.2%	86.8%	93.2%	83.0%	59.2%	9.7%	27.0%	8.8%	7.0%	7.4%	10.3%	-1.1%			
	DK/NA	7.4%	1.6%	1.6%	5.9%	1.8%	14.7%	16.8%	4.0%	0.0%	0.0%	1.5%	3.0%	7.5%	10.8%	-3.4%	-1.6%	-1.6%	-4.4%	1.2%	-7.1%	-6.0%			
There should be a global target to stop the use of antibiotics for growth promotion and infection prevention without veterinary advice	Mean	7.96	7.80	7.37	7.58	8.26	8.25	8.43	8.33	8.21	7.90	7.99	8.55	8.71	8.54	0.37***	0.41*	0.53**	0.41**	0.29	0.46**	0.11			
	Oppose	6.0%	3.3%	7.3%	7.2%	4.6%	7.8%	5.2%	3.1%	1.2%	4.9%	2.3%	2.2%	4.1%	3.7%	-2.9%	-2.1%	-2.5%	-4.9%	-2.5%	-3.7%	-1.4%			
	In the middle	8.7%	12.2%	9.0%	12.0%	5.5%	7.2%	7.6%	7.6%	8.6%	5.2%	12.8%	6.1%	4.2%	10.8%	-1.1%	-3.6%	-3.8%	0.7%	0.6%	-3.0%	3.2%			
	Support	80.5%	83.2%	83.6%	75.8%	83.1%	76.9%	81.3%	85.6%	90.1%	87.1%	83.8%	88.9%	83.6%	80.2%	5.1%	7.0%	3.4%	8.0%	5.8%	6.7%	-1.1%			
	DK/NA	4.8%	1.3%	0.0%	5.0%	6.8%	8.1%	5.9%	3.6%	0.0%	2.9%	1.1%	2.8%	8.1%	5.2%	-1.1%	-1.3%	2.9%	-3.8%	-3.9%	0.0%	-0.7%			
Antibiotic resistance should be included as a cause of death on all national death certificates to enable collection of data on mortality.	Mean	7.91	7.01	7.41	7.74	8.32	8.67	8.06	8.51	8.52	8.02	7.98	8.63	9.14	8.62	0.60***	1.52***	0.61***	0.24	0.31*	0.47***	0.56*			
	Oppose	4.9%	11.4%	3.7%	1.5%	4.2%	2.8%	7.7%	2.8%	3.3%	1.9%	1.9%	2.4%	1.4%	6.9%	-2.1%	-8.0%	-1.8%	0.5%	-1.8%	-1.4%	-0.8%			
	In the middle	8.8%	15.1%	11.0%	13.9%	3.4%	4.7%	7.8%	5.4%	4.6%	7.1%	10.8%	4.5%	3.6%	2.8%	-3.4%	-10.6%	-3.9%	-3.1%	1.1%	-1.1%	-5.0%			
	Support	80.8%	71.1%	83.7%	80.2%	86.8%	83.3%	76.4%	88.9%	92.1%	91.0%	85.6%	89.8%	88.2%	86.8%	8.1%	21.1%	7.3%	5.4%	3.1%	4.9%	10.3%			
	DK/NA	5.5%	2.4%	1.7%	4.4%	5.7%	9.2%	8.1%	2.9%	0.0%	0.0%	1.7%	3.3%	6.8%	3.6%	-2.7%	-2.4%	-1.7%	-2.7%	-2.4%	-2.4%	-4.5%			

Using Likert score, percentage levels of support, opposition, those ‘in the middle’ and those responding ‘Don’t Know or Non-Applicable’ (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by * $p < 0.05$, ** $p < 0.01$, *** $p < 0.001$ using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 17: Surveillance- mean ratings for each proposal

Proposal	Pre										Post						Difference							
	Measure	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR	All	TZ	NG	ID	IN	CO	BR		
National surveillance programs should be in place to monitor - infection rates, bacterial types, antibiotic use, and antibiotic resistance – in humans, animals (including fish) and water systems.	Mean	8.42	7.95	7.98	8.20	8.54	8.94	8.77	8.82	8.69	8.46	8.18	8.95	9.25	9.30	0.40***	0.73***	0.48***	-0.02	0.41***	0.31**	0.53***		
	Oppose	2.2%	5.5%	3.9%	0.3%	2.1%	0.4%	1.9%	0.8%	1.7%	0.6%	1.0%	0.5%	0.3%	1.0%	-1.4%	-3.7%	-3.4%	0.6%	-1.6%	-0.1%	-0.9%		
	In the middle	6.9%	18.6%	2.6%	10.1%	3.2%	4.2%	5.2%	3.0%	2.7%	0.2%	8.5%	1.1%	2.4%	3.9%	-3.9%	-16.0%	-2.5%	-1.6%	-2.0%	-1.8%	-1.3%		
	Support	87.8%	73.7%	93.4%	87.8%	93.2%	86.7%	90.5%	94.4%	95.6%	99.3%	88.8%	96.9%	95.2%	89.5%	6.6%	21.8%	5.8%	1.1%	3.7%	8.5%	-1.0%		
	DK/NA	3.1%	2.2%	0.0%	1.8%	1.6%	8.8%	2.4%	1.8%	0.0%	0.0%	1.8%	1.5%	2.2%	5.5%	-1.3%	-2.2%	0.0%	-0.1%	-0.1%	-6.6%	3.1%		
International funding support should be available for surveillance in countries willing to invest in laboratory capacity, monitoring, and reporting and where systems are not yet available.	Mean	8.36	7.58	8.01	8.14	8.44	8.99	8.80	8.77	8.42	8.65	7.98	8.85	9.31	9.31	0.42***	0.85***	0.65***	-0.16	0.41***	0.31***	0.50***		
	Oppose	3.5%	16.6%	1.0%	1.6%	2.5%	0.8%	0.6%	1.2%	2.0%	0.0%	2.7%	0.9%	1.2%	0.2%	-2.3%	-14.6%	-1.0%	1.1%	-1.6%	0.4%	-0.4%		
	In the middle	6.4%	8.9%	2.3%	9.7%	7.6%	3.5%	7.8%	3.8%	6.4%	1.3%	10.2%	2.4%	1.1%	2.8%	-2.6%	-2.5%	-1.0%	0.5%	-5.2%	-2.4%	-5.0%		
	Support	86.9%	74.4%	96.6%	86.5%	87.6%	90.1%	83.3%	93.8%	91.6%	97.7%	86.0%	96.5%	95.5%	94.3%	6.9%	17.1%	1.1%	-0.5%	8.9%	5.5%	11.0%		
	DK/NA	3.2%	0.1%	0.0%	2.2%	2.3%	5.6%	8.3%	1.2%	0.0%	1.0%	1.1%	0.2%	2.2%	2.7%	-2.0%	-0.1%	1.0%	-1.1%	-2.2%	-3.5%	-5.6%		
Surveillance data should be reported nationally, regionally, and to the World Health Organization, Food and Agriculture Organisation, and the World Organisation for Animal Health.	Mean	8.31	7.74	7.79	8.05	8.41	8.83	8.92	8.72	8.37	8.27	8.23	8.92	9.15	9.27	0.41***	0.63***	0.48***	0.18	0.51***	0.32**	0.35		
	Oppose	1.9%	2.5%	3.1%	2.5%	2.9%	0.6%	0.0%	1.0%	1.9%	0.0%	0.4%	1.1%	1.8%	0.2%	-0.9%	-0.6%	-3.1%	-2.1%	-1.8%	1.2%	0.2%		
	In the middle	8.6%	23.9%	6.1%	9.8%	4.8%	5.2%	4.7%	3.0%	5.0%	2.6%	7.5%	1.3%	1.3%	1.6%	-5.5%	-18.9%	-3.5%	-2.3%	-3.5%	-3.9%	-3.1%		
	Support	86.5%	73.6%	90.9%	85.5%	87.8%	91.0%	86.9%	94.3%	93.1%	96.5%	90.9%	97.0%	93.0%	95.3%	7.8%	19.5%	5.6%	5.4%	9.1%	2.0%	8.4%		
	DK/NA	3.1%	0.0%	0.0%	2.2%	4.5%	3.3%	8.4%	1.7%	0.0%	1.0%	1.1%	0.7%	3.9%	2.9%	-1.4%	0.0%	1.0%	-1.0%	-3.8%	0.6%	-5.5%		
Laboratory systems for measuring antibiotic resistance in bacteria for humans and animals should be integrated with shared One Health resources.	Mean	8.31	7.90	7.78	8.19	8.41	8.85	8.75	8.59	8.74	8.30	8.19	8.60	9.00	8.72	0.28***	0.84***	0.52***	0.00	0.19	0.15	-0.03		
	Oppose	1.9%	5.8%	1.2%	1.6%	2.4%	0.6%	0.8%	1.5%	4.2%	0.9%	1.7%	0.7%	1.2%	1.0%	-0.4%	-1.5%	-0.4%	0.1%	-1.7%	0.5%	0.2%		
	In the middle	5.8%	6.7%	4.3%	8.6%	6.0%	5.3%	3.8%	4.3%	3.0%	1.5%	6.5%	4.2%	4.1%	6.9%	-1.4%	-3.7%	-2.8%	-2.1%	-1.8%	-1.3%	3.2%		
	Support	85.4%	87.3%	94.0%	87.6%	87.9%	73.1%	87.1%	89.8%	91.4%	97.7%	90.1%	92.1%	82.3%	87.6%	4.4%	4.1%	3.7%	2.5%	4.2%	9.2%	0.5%		
	DK/NA	6.9%	0.3%	0.5%	2.2%	3.7%	21.0%	8.4%	4.3%	1.4%	0.0%	1.7%	2.9%	12.5%	4.5%	-2.5%	1.1%	-0.5%	-0.5%	-0.8%	-8.5%	-3.8%		
Data from point of care rapid diagnostic tests should be incorporated into routine national surveillance reporting.	Mean	8.26	7.77	7.70	8.11	8.40	8.60	8.88	8.65	8.65	8.33	8.24	8.62	9.00	9.00	0.39***	0.88***	0.63***	0.13	0.23	0.39***	0.12		
	Oppose	2.3%	5.0%	2.2%	2.0%	2.8%	2.2%	0.0%	1.4%	2.0%	1.5%	0.9%	2.0%	1.5%	0.4%	-0.9%	-3.1%	-0.7%	-1.1%	-0.9%	-0.7%	0.4%		
	In the middle	6.0%	8.7%	2.9%	9.1%	4.1%	5.6%	6.7%	4.4%	4.9%	2.7%	7.9%	2.3%	3.4%	6.3%	-1.6%	-3.8%	-0.3%	-1.2%	-1.7%	-2.2%	-0.4%		
	Support	86.8%	85.8%	92.6%	86.7%	85.9%	82.4%	89.5%	92.3%	92.5%	95.8%	90.1%	92.8%	91.9%	90.4%	5.4%	6.8%	3.2%	3.4%	6.9%	9.6%	0.8%		
	DK/NA	4.8%	0.5%	2.3%	2.3%	7.2%	9.8%	3.8%	1.9%	0.6%	0.0%	1.1%	2.9%	3.2%	2.9%	-2.9%	0.1%	-2.3%	-1.1%	-4.3%	-6.7%	-0.9%		

Using Likert score, percentage levels of support, opposition, those ‘in the middle’ and those responding ‘Don’t Know or Non-Applicable’ (DK/NA) are shown pre and post deliberation. Statistical significance between levels of support pre- and post-deliberation are expressed by *p<0.05, **p<0.01, ***p<0.001 using the paired T-Test. Countries are represented by the following abbreviations: Tanzania (TZ), Nigeria (NG), Indonesia (ID), India (IN), Colombia (CO) and Brazil (BR).

Table 18: Knowledge Questions

Question	Expert Answer
What does antibiotic resistance mean? Choose only one answer	a. Medicine that is working against your body to make you more sick b. A virus causing infection in your body is able to resist the action of an antibiotic c. Antibiotic manufacturers have become resistant to producing antibiotics d. Bacteria causing an infection are able to resist the action of an antibiotic* e. Don't know
When was the latest discovery of a new antibiotic class that has reached the market?	a. 2019 b. 2005 c. 1999 d. 1987* e. 1956 f. Don't Know
Which of the following statements are TRUE about antibiotics? Choose only one answer.	a. Antibiotics can kill bacteria. b. Antibiotics have side-effects such as diarrhea, nausea and vomiting. c. Antibiotics cannot kill viruses. d. People can be allergic to antibiotics. e. All of the above* f. Don't know
Which of the following is a way to prevent infection, in order to reduce antibiotic resistance? Choose only one answer	a. Vaccination of food production animals b. Wash hands c. Vaccination of humans against influenza d. Hygienic food preparation e. All of the above* f. Don't know
What does the term 'superbug' mean?	a. One of the largest bugs, in size, on earth that causes the worse colds b. A bug with 'super' abilities to eat large amount of bacteria to cure illnesses c. The bacteria that can be killed by any antibiotic d. The most resistant types of bacteria almost always found in hospitals* e. Don't know
Which of the following statements is a fact about antibiotic resistance	a. More children under the age of 5 years die from lack of access to an antibiotic than die of antibiotic resistant bacterial infection

	b. Antibiotic use has been associated with obesity later in life c. Antibiotic use has been associated with developing inflammatory bowel disease (ulcerative colitis and Crohn's Disease) in later life d. Antibiotic misuse in humans is a major cause of increasing antibiotic resistance e. All of the above* f. Don't Know
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*denotes the correct answer to each question as ascertained by the Expert Advisory Panel

Table 19: Differences between the intervention group changes and the control group changes before and after deliberation for each of the 45 proposals for each country. The p-value column indicates the significance of the analysis for each row, comparison of the treatment and control group for the particular question.

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Brazil	p01	178	188	9.23	9.462	-1.482	0.139	0.287
Brazil	p02	178	188	8.869	8.955	-0.447	0.655	0.808
Brazil	p03	178	188	7.162	6.884	0.793	0.428	0.612
Brazil	p04	178	188	6.158	6.544	-1.042	0.298	0.473
Brazil	p05	178	188	7.207	7.052	0.49	0.624	0.788
Brazil	p06	178	188	6.407	6.693	-0.793	0.429	0.612
Brazil	p07	178	188	7.919	7.815	0.369	0.712	0.851
Brazil	p08	178	188	8.469	8.744	-1.328	0.185	0.34
Brazil	p09	178	188	8.337	8.742	-1.588	0.113	0.247
Brazil	p10	178	188	8.343	8.486	-0.625	0.532	0.711
Brazil	p11	178	188	8.328	8.569	-1.05	0.295	0.471
Brazil	p12	178	188	9.318	9.188	0.898	0.37	0.558
Brazil	p13	178	188	8.949	9.028	-0.468	0.64	0.798
Brazil	p14	178	188	9.264	9.311	-0.349	0.727	0.858
Brazil	p15	178	188	9.568	9.647	-0.726	0.469	0.639
Brazil	p16	178	188	9.808	9.786	0.301	0.763	0.882
Brazil	p17	178	188	9.631	9.604	0.235	0.814	0.916
Brazil	p18	178	188	9.384	9.524	-0.899	0.369	0.558
Brazil	p19	178	188	9.386	9.333	0.365	0.715	0.851
Brazil	p20	178	188	9.394	9.5	-0.765	0.445	0.629
Brazil	p21	178	188	9.186	9.112	0.434	0.664	0.815
Brazil	p22	178	188	8.865	8.852	0.059	0.953	0.984
Brazil	p23	178	188	9.491	9.387	0.777	0.438	0.622
Brazil	p24	178	188	8.563	8.584	-0.084	0.933	0.977
Brazil	p25	178	188	9.52	9.538	-0.148	0.882	0.97
Brazil	p26	178	188	9.042	8.966	0.38	0.704	0.845
Brazil	p27	178	188	8.481	8.454	0.102	0.919	0.977
Brazil	p28	178	188	7.974	7.525	1.425	0.155	0.304
Brazil	p29	178	188	9.198	9.193	0.028	0.978	0.992
Brazil	p30	178	188	9.351	9.486	-1.037	0.301	0.475
Brazil	p31	178	188	7.247	6.477	2.023	0.044	0.116
Brazil	p32	178	188	9.036	8.988	0.263	0.792	0.906
Brazil	p33	178	188	9.44	9.44	-0.002	0.999	1

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Brazil	p34	178	188	9.224	9.3	-0.466	0.641	0.798
Brazil	p35	178	188	9.08	9.033	0.242	0.809	0.914
Brazil	p36	178	188	9.219	9.074	0.874	0.383	0.571
Brazil	p37	178	188	9.285	9.08	1.187	0.236	0.403
Brazil	p38	178	188	7.975	7.742	0.736	0.462	0.636
Brazil	p39	178	188	8.795	8.682	0.501	0.617	0.782
Brazil	p40	178	188	8.524	8.415	0.417	0.677	0.827
Brazil	p41	178	188	9.099	8.978	0.745	0.457	0.636
Brazil	p42	178	188	8.964	8.948	0.094	0.926	0.977
Brazil	p43	178	188	9.121	9.073	0.301	0.763	0.882
Brazil	p44	178	188	9.073	8.905	0.972	0.332	0.512
Brazil	p45	178	188	9.198	9.116	0.535	0.593	0.766
Colombia	p01	185	275	8.797	8.996	-1.143	0.254	0.426
Colombia	p02	185	275	8.916	8.715	1.201	0.231	0.399
Colombia	p03	185	275	7.415	7.516	-0.387	0.699	0.843
Colombia	p04	185	275	6.983	6.996	-0.044	0.965	0.987
Colombia	p05	185	275	7.809	7.682	0.524	0.601	0.769
Colombia	p06	185	275	7.186	7.181	0.019	0.985	0.993
Colombia	p07	185	275	8.797	8.478	1.826	0.069	0.166
Colombia	p08	185	275	8.347	8.265	0.405	0.686	0.83
Colombia	p09	185	275	8.807	8.582	1.127	0.261	0.432
Colombia	p10	185	275	8.712	8.606	0.612	0.541	0.719
Colombia	p11	185	275	8.709	8.521	1.087	0.278	0.455
Colombia	p12	185	275	9.078	8.885	1.429	0.154	0.303
Colombia	p13	185	275	8.594	8.652	-0.334	0.738	0.867
Colombia	p14	185	275	8.966	9.065	-0.669	0.504	0.683
Colombia	p15	185	275	9.486	9.52	-0.296	0.767	0.882
Colombia	p16	185	275	9.723	9.787	-1.001	0.317	0.495
Colombia	p17	185	275	9.568	9.581	-0.145	0.885	0.97
Colombia	p18	185	275	9.525	9.525	0	1	1
Colombia	p19	185	275	9.238	9.23	0.053	0.958	0.984
Colombia	p20	185	275	9.17	9.151	0.116	0.908	0.977
Colombia	p21	185	275	9.211	9.199	0.101	0.919	0.977

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Colombia	p22	185	275	8.452	7.908	2.343	0.02	0.059
Colombia	p23	185	275	9.05	9.063	-0.094	0.925	0.977
Colombia	p24	185	275	7.814	7.742	0.246	0.806	0.914
Colombia	p25	185	275	9.244	9.311	-0.47	0.639	0.798
Colombia	p26	185	275	9.135	8.755	2.417	0.016	0.051
Colombia	p27	185	275	8.679	8.457	1.059	0.29	0.471
Colombia	p28	185	275	8.415	8.403	0.059	0.953	0.984
Colombia	p29	185	275	9.12	8.865	1.706	0.089	0.207
Colombia	p30	185	275	8.989	9.099	-0.796	0.426	0.612
Colombia	p31	185	275	7.904	7.884	0.076	0.939	0.979
Colombia	p32	185	275	9.235	9.011	1.665	0.097	0.217
Colombia	p33	185	275	9.374	9.158	1.68	0.094	0.212
Colombia	p34	185	275	9.201	9.098	0.81	0.418	0.607
Colombia	p35	185	275	8.983	8.996	-0.084	0.933	0.977
Colombia	p36	185	275	9.006	8.881	0.747	0.456	0.636
Colombia	p37	185	275	8.943	9.085	-0.903	0.367	0.558
Colombia	p38	185	275	8.47	8.533	-0.299	0.765	0.882
Colombia	p39	185	275	8.462	8.44	0.098	0.922	0.977
Colombia	p40	185	275	8.729	8.6	0.666	0.506	0.683
Colombia	p41	185	275	9.029	8.984	0.299	0.765	0.882
Colombia	p42	185	275	9	9.077	-0.555	0.579	0.759
Colombia	p43	185	275	8.988	8.903	0.542	0.588	0.766
Colombia	p44	185	275	8.794	8.819	-0.14	0.889	0.97
Colombia	p45	185	275	8.653	8.66	-0.038	0.97	0.988
India	p01	160	231	8.558	8.827	-1.492	0.137	0.284
India	p02	160	231	8.189	8.726	-2.823	0.005	0.021
India	p03	160	231	7.955	7.715	1.002	0.317	0.495
India	p04	160	231	7.297	7.441	-0.528	0.598	0.769
India	p05	160	231	7.316	7.61	-1.156	0.249	0.422
India	p06	160	231	7.182	7.53	-1.377	0.169	0.317
India	p07	160	231	8.057	8.173	-0.555	0.579	0.759
India	p08	160	231	8.195	8.453	-1.448	0.149	0.298
India	p09	160	231	8.19	8.538	-1.685	0.093	0.212

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
India	p43	160	231	8.25	8.456	-1.128	0.26	0.432
India	p44	160	231	8.196	8.302	-0.579	0.563	0.745
India	p45	160	231	8.238	8.335	-0.515	0.607	0.773
Indonesia	p01	200	198	8.53	8.204	2.222	0.027	0.076
Indonesia	p02	200	198	8.388	8.021	2.311	0.021	0.062
Indonesia	p03	200	198	8.219	7.661	3.061	0.002	0.011
Indonesia	p04	200	198	7.909	7.13	3.719	0	0.002
Indonesia	p05	200	198	7.855	6.853	4.872	0	0
Indonesia	p06	200	198	8.057	7.222	4.31	0	0
Indonesia	p07	200	198	8.236	7.969	1.503	0.134	0.28
Indonesia	p08	200	198	8.25	7.791	2.514	0.012	0.042
Indonesia	p09	200	198	7.69	6.547	4.97	0	0
Indonesia	p10	200	198	8.327	7.912	2.575	0.01	0.039
Indonesia	p11	200	198	8.411	7.809	3.663	0	0.002
Indonesia	p12	200	198	8.497	8.231	1.709	0.088	0.207
Indonesia	p13	200	198	8.382	8	2.37	0.018	0.056
Indonesia	p14	200	198	8.452	8.308	0.878	0.38	0.571
Indonesia	p15	200	198	8.894	8.602	1.935	0.054	0.139
Indonesia	p16	200	198	8.726	8.73	-0.024	0.981	0.992
Indonesia	p17	200	198	8.754	8.566	1.278	0.202	0.352
Indonesia	p18	200	198	8.709	8.497	1.376	0.17	0.317
Indonesia	p19	200	198	8.601	8.062	3.325	0.001	0.005
Indonesia	p20	200	198	8.585	8.223	2.313	0.021	0.062
Indonesia	p21	200	198	8.575	8.39	1.284	0.2	0.352
Indonesia	p22	200	198	8.665	8.485	1.28	0.201	0.352
Indonesia	p23	200	198	8.566	8.346	1.566	0.118	0.256
Indonesia	p24	200	198	8.602	8.219	2.569	0.011	0.039
Indonesia	p25	200	198	8.621	8.699	-0.539	0.59	0.766
Indonesia	p26	200	198	8.032	7.438	3.149	0.002	0.008
Indonesia	p27	200	198	7.689	6.973	3.475	0.001	0.004
Indonesia	p28	200	198	8.061	7.742	1.851	0.065	0.161
Indonesia	p29	200	198	8.402	8.114	1.993	0.047	0.123
Indonesia	p30	200	198	8.425	8.234	1.333	0.183	0.339

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Indonesia	p31	200	198	8.062	7.387	3.458	0.001	0.004
Indonesia	p32	200	198	8.551	8.338	1.46	0.145	0.297
Indonesia	p33	200	198	8.422	8.292	0.864	0.388	0.576
Indonesia	p34	200	198	8.308	7.826	2.656	0.008	0.032
Indonesia	p35	200	198	8.317	7.885	2.737	0.007	0.026
Indonesia	p36	200	198	8.396	7.995	2.675	0.008	0.031
Indonesia	p37	200	198	8.416	8.216	1.317	0.189	0.344
Indonesia	p38	200	198	8.077	7.942	0.837	0.403	0.592
Indonesia	p39	200	198	8.137	7.676	2.595	0.01	0.037
Indonesia	p40	200	198	8.169	7.811	2.278	0.023	0.066
Indonesia	p41	200	198	8.422	8.215	1.395	0.164	0.315
Indonesia	p42	200	198	8.355	8.16	1.289	0.198	0.352
Indonesia	p43	200	198	8.34	8.092	1.552	0.121	0.26
Indonesia	p44	200	198	8.372	8.216	1.05	0.294	0.471
Indonesia	p45	200	198	8.32	8.124	1.288	0.199	0.352
Nigeria	p01	210	203	7.608	8.158	-3.078	0.002	0.01
Nigeria	p02	210	203	7.364	8.005	-3.618	0	0.002
Nigeria	p03	210	203	7.268	7.97	-3.53	0	0.003
Nigeria	p04	210	203	6.895	7.724	-4.041	0	0.001
Nigeria	p05	210	203	7.25	7.851	-3.194	0.002	0.007
Nigeria	p06	210	203	6.781	7.411	-2.962	0.003	0.014
Nigeria	p07	210	203	7.558	7.98	-2.371	0.018	0.056
Nigeria	p08	210	203	7.584	7.901	-1.871	0.062	0.155
Nigeria	p09	210	203	7.795	8.227	-2.536	0.012	0.041
Nigeria	p10	210	203	7.55	7.881	-1.973	0.049	0.128
Nigeria	p11	210	203	7.373	7.975	-3.349	0.001	0.005
Nigeria	p12	210	203	7.622	8.286	-3.955	0	0.001
Nigeria	p13	210	203	7.216	7.975	-4.227	0	0
Nigeria	p14	210	203	7.524	8.31	-4.529	0	0
Nigeria	p15	210	203	7.699	8.345	-3.677	0	0.002
Nigeria	p16	210	203	8.351	8.673	-1.824	0.069	0.166
Nigeria	p17	210	203	8.033	8.655	-3.907	0	0.001
Nigeria	p18	210	203	7.928	8.302	-2.201	0.028	0.078

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Nigeria	p19	210	203	7.584	8	-2.434	0.015	0.05
Nigeria	p20	210	203	7.629	7.851	-1.38	0.168	0.317
Nigeria	p21	210	203	7.856	8.188	-1.92	0.056	0.142
Nigeria	p22	210	203	7.962	8	-0.213	0.831	0.926
Nigeria	p23	210	203	7.5	8.207	-3.952	0	0.001
Nigeria	p24	210	203	6.77	7.76	-4.897	0	0
Nigeria	p25	210	203	7.867	8.404	-3.034	0.003	0.011
Nigeria	p26	210	203	7.433	7.837	-2.412	0.016	0.051
Nigeria	p27	210	203	7.053	7.627	-3.147	0.002	0.008
Nigeria	p28	210	203	6.986	7.667	-3.484	0.001	0.004
Nigeria	p29	210	203	7.274	8.015	-4.216	0	0
Nigeria	p30	210	203	7.385	7.915	-2.935	0.004	0.015
Nigeria	p31	210	203	6.626	7.41	-3.528	0	0.003
Nigeria	p32	210	203	7.337	7.921	-3.348	0.001	0.005
Nigeria	p33	210	203	7.833	8.379	-3.295	0.001	0.005
Nigeria	p34	210	203	7.59	8.276	-4.375	0	0
Nigeria	p35	210	203	7.519	8.084	-3.411	0.001	0.004
Nigeria	p36	210	203	7.469	7.837	-2.124	0.034	0.093
Nigeria	p37	210	203	7.21	8.153	-5.106	0	0
Nigeria	p38	210	203	7.082	7.495	-2.028	0.043	0.115
Nigeria	p39	210	203	6.883	7.419	-2.463	0.014	0.046
Nigeria	p40	210	203	6.805	7.62	-4.332	0	0
Nigeria	p41	210	203	7.172	8.054	-5.132	0	0
Nigeria	p42	210	203	7.595	8.069	-2.88	0.004	0.017
Nigeria	p43	210	203	7.584	8.005	-2.319	0.021	0.062
Nigeria	p44	210	203	7.428	7.841	-2.494	0.013	0.044
Nigeria	p45	210	203	7.177	7.79	-3.535	0	0.003
Tanzania	p01	206	185	9.146	8.173	5.318	0	0
Tanzania	p02	206	185	8.771	7.876	4.201	0	0
Tanzania	p03	206	185	7.965	8.027	-0.226	0.821	0.92
Tanzania	p04	206	185	6.736	6.071	1.843	0.066	0.162
Tanzania	p05	206	185	7.692	7.73	-0.137	0.891	0.97
Tanzania	p06	206	185	5.827	6.168	-0.923	0.357	0.547

Country	Proposal	Control n	Participant n	Control Mean	Participant Mean	Statistic	p-value	Adjusted p
Tanzania	p07	206	185	8.5	8.039	1.901	0.058	0.146
Tanzania	p08	206	185	8.292	7.549	2.984	0.003	0.013
Tanzania	p09	206	185	7.765	7.827	-0.21	0.834	0.926
Tanzania	p10	206	185	9.02	8.038	5.236	0	0
Tanzania	p11	206	185	9.064	8.097	5.329	0	0
Tanzania	p12	206	185	9.01	7.995	5.173	0	0
Tanzania	p13	206	185	8.626	8.032	2.886	0.004	0.017
Tanzania	p14	206	185	8.777	7.956	4.082	0	0.001
Tanzania	p15	206	185	8.99	8.272	3.656	0	0.002
Tanzania	p16	206	185	9.185	8.357	4.483	0	0
Tanzania	p17	206	185	8.941	8.103	4.155	0	0
Tanzania	p18	206	185	9.108	7.842	5.928	0	0
Tanzania	p19	206	185	8.789	7.95	3.867	0	0.001
Tanzania	p20	206	185	8.852	7.579	5.6	0	0
Tanzania	p21	206	185	8.736	8.059	3.303	0.001	0.005
Tanzania	p22	206	185	8.754	8.027	3.47	0.001	0.004
Tanzania	p23	206	185	9.025	7.989	5.15	0	0
Tanzania	p24	206	185	8.354	7.503	3.391	0.001	0.004
Tanzania	p25	206	185	8.762	8.295	2.308	0.022	0.062
Tanzania	p26	206	185	8.387	8.065	1.441	0.15	0.299
Tanzania	p27	206	185	8.128	7.743	1.631	0.104	0.229
Tanzania	p28	206	185	8.106	7.558	2.214	0.027	0.076
Tanzania	p29	206	185	8.168	7.541	2.499	0.013	0.043
Tanzania	p30	206	185	8.541	7.636	3.863	0	0.001
Tanzania	p31	206	185	7.608	7.114	1.78	0.076	0.181
Tanzania	p32	206	185	8.803	8.12	3.397	0.001	0.004
Tanzania	p33	206	185	8.932	8.151	3.902	0	0.001
Tanzania	p34	206	185	8.877	8.2	3.414	0.001	0.004
Tanzania	p35	206	185	8.545	8.049	2.352	0.019	0.058
Tanzania	p36	206	185	8.562	8.017	2.56	0.011	0.039
Tanzania	p37	206	185	8.835	8.13	3.406	0.001	0.004
Tanzania	p38	206	185	8.277	7.566	3.088	0.002	0.01
Tanzania	p39	206	185	7.819	7.797	0.09	0.928	0.977
Tanzania	p40	206	185	8.547	7.115	5.966	0	0
Tanzania	p41	206	185	8.824	7.918	4.386	0	0
Tanzania	p42	206	185	8.467	7.908	2.552	0.011	0.039
Tanzania	p43	206	185	8.747	7.876	4.387	0	0
Tanzania	p44	206	185	8.264	7.725	2.191	0.029	0.079
Tanzania	p45	206	185	8.454	7.875	2.641	0.009	0.033